



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : مرفت عبد اللطيف سليم عبد المتعال
المحافظة : القاهرة الحي / المدينة : المقطم

العنوان : 0
تاريخ تسجيل الإستمارة : 26-09-2026

رقم البطاقة : 25907111300062 رقم التليفون : 01235425163
العمر (تاريخ الميلاد) : 11-07-1959 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : ورم خبيث بالمستقيم
نتائج الأبحاث : (مرفق)

التشخيص النهائي : ورم خبيث بالمستقيم
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة
قسم العلاج : الاورام [11129]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: مرفت عبد اللطيف سليم عبد المتعال National ID: 25907111300062	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

A patient of colorectal cancer for Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
مملكة تقيس الشخصية

مرقت
عبد اللطيف سليم عبد المتعال
٥١٦٠
المقطم - القاهرة

٢٥٩.٧١١٣.٠٠٠.٦٢



LC4837863



Patient Name : الأستاذة / مرفت عبداللطيف سليم	Request No. : 13007630
Patient No. : 25907111300062	Request Date : 12/08/2023
Path No. : 2623/23	Verification Date : 21/08/2023
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 64 Y	Department : Out Patient
Referreing Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Bilateral breast cancer, post NACT.
Nature Specimen: Right WLE and Left modified radical mastectomy specimen.

Gross Examination:

Right breast :

* Right WLE specimen measured 7x6cm, with covering skin ellipse measured 3x1.5 cm , cut section revealed fairly circumscribed grayish white rubbery lobulated mass , 0.5 cm from deep margin .

Left breast :

- * Left MRM measured 26x22x6 cm with covering skin ellipse measured 19x10 cm.
- * Tumor extent: A greyish white firm tumor bed site.
- * Size: 10x5x4 cm.
- * Site : Central .
- * Skin involvement: Nipple & areola were unremarkable grossly.
- * Distance from margins: 2cm away from deep margin.
- * Axilla measured 7x7 cm entagling 11 L.Ns, measured up to 1.2cm in diameter.

Microscopic Examination:

Right breast :

Right breast, WLE :

-Histopathological examination reveals a benign growth made up of proliferating small patent mammary ducts. Some of the ducts are compressed and closed, other are patent. The ducts are lined by inner columnar and outer myoepithelial cells. The stroma in between is formed of loose fibrous connective tissue, associated with diffuse stromal fibrosis, focal adenosis , irregular and cystically dilated ducts, and ducts showing apocrine metaplasia. The ducts are lined by inner columnar and outer myoepithelial cells.

Verified by :

Dr.Marwa mahmoud



Patient Name : الأستاذة / مرفت عبداللطيف سليم	Request No. : 13007630
Patient No. : 25907111300062	Request Date : 12/08/2023
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Age : 64 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Left breast :

- Total tumor regression
- Complete pathological response .
- There is hyalinosis , calcifications , and histiocytic aggregats , rest of the breast showed fibrocystic changes and fibroadenosis .
- * Skin involvement: Free Nipple & areola.
- * Lymph node involvement: Free (0/9) , one lymph node showed microcalcification and hyalinosis .
- * Resection Margins: Free deep margin .

Diagnosis:

*1- Right breast , WLE;

Fibroadenoma and fibrocystic changes, No malignancy.

*2- Left breast cancer, post NACT/ MRM specimen:✓

- Total tumor regression in a previously diagnosed carcinoma.
- Complete pathological response.
- Free Deep margin .
- Free nipple and areola .
- Free LNs (0/9) .
- Stage: ypT0, ypN0.

Signature

Prof. Lobna Omar

Dr. Marwa Mahmoud

Verified by :

Dr.Marwa mahmoud

Printing Date : 23/08/2023 09:41:49

Page 2 of 3

Printing By : Ali mostafa

جمهورية مصر العربية
بطاقة تحقيق الشخصية

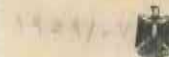


مرفقة

عبداللطيف سليم عبدالمتعال

قطعه ٥١٦٠

المقطم - القاهرة



٢٥٩٠٧١١١٣٠٠٠

LC4837863

National ID Front
IDOC TYPE ID_FRONT



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ربة منزل

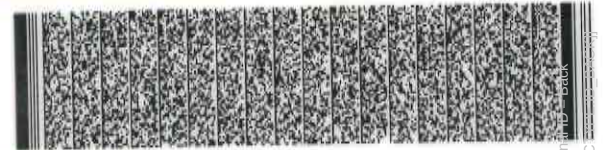
ارملة

مسألة

افشى



البطاقة سارية حتى ٢٠٣٢/٠٤/١٥





Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : مرفت عبداللطيف سليم عبدالمتعال Sex:

3- Medical number:

Male

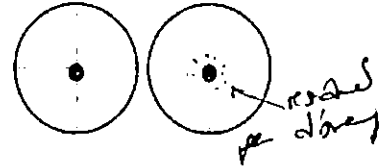
83123313

Old Med :

359/2023

2-ID number: 25907111330062

Patient Age : 64 Y



Family history

Phone

01006333193

BMI

REASON OF MDT

POST NACT

marital status

NRS Score

GROUP

A

Date of MDT

2023-07-19



Co morbidity

DM - H1A

Menopausal
status

POSTMENOPAUSAL

Clinical data

Breast Imaging :

LT UOQ 70*3.5*5.0 CM with intraductal extension
rt oval shaped lesion

Mets Workup :

c.t : free

BX:

ER:

0/8

PR:

0/8

Her 2 Neu :

3

SISH :

Ki67 :

30%

LT IDC GII

+VE

T

4D

N

1

M

0

Sonomammography

RT UOQ UIO FIBROADENOMA

LT UOQ 70*3.5*5.0 CM with intraductal extension

rt oval shaped lesion

PATIENT HISTORY

LABC-->HR-VE,,HER2 +VE

FINISHED TCX18 WK+HPX6

SM PENDING

FOR YOUR KIND CARE



ERM036727200

MDT OUTCOME & PLAN

for LT MRM & RT lumpectomy as per initial MDT decision

SIGNATURE

MEDICAL ONCOLOGY

د/ شيماء لاشين

RADIOLOGY

د/ نشور عبداللطيف

SURGERY

د/ محمد عبدالرحمن

PATHOLOGY

Psychiatric

Nutrition

MRS: Mervat Abd Al Latif Selim	Date: 2023/07/18
Dear. SIR.	ID: 928155

BILATERAL DIGITAL COMPUTED MAMMOGRAPHY AND SONOGRAPHY REVEALED:-

Known patient of left breast carcinoma on neo adjuvant CTH, the current study revealed:

The right breast:

- The right breast shows normal nipple, areola, skin and subcutaneous tissue thickness.
- It shows condensation of glandular tissue elements, with nodularity, suggestive of fibroadenosis.
- The right breast shows paraareolar inner quadrant 4 O'clock position lobulated opacity **by ultrasonography** it proved to be lobulated soft tissue lesion at 5 O'clock position measuring 17 x 10 mm, mostly represent fibroadenoma.
- The right breast shows few simple cysts measuring 3 x 2 mm.
- No suspicious breast masses are detected.
- No malignant micro calcifications are detected.
- No suspicious right axillary lymphadenopathy.

The left breast:

- The left breast shows mild diffuse skin and subcutaneous soft tissue thickening and edema.
- It shows condensation of glandular tissue elements, with nodularity, suggestive of fibroadenosis.
- It shows outer quadrant diffuse suspicious micro calcifications, **by ultrasonography** it proved to be area of underlying fibrosis and architectural distortion with suspicious mass lesion at 2 O'clock position now it measures 16 x 9 mm (for comparison with the previous study).

ا.د. أحمد كمال الدري
ا.د. محمد أبو الهدى درويش
ا.د. عمر حسين عمر
ا.د. وحيد حسين طنطاوى
أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

MRS: Mervat Abd Al Latif Selim	Date: 2023/07/18
Dear. SIR.	ID: 928155

- The left breast shows upper central retroareolar benign looking opacity with benign looking coarse calcifications inside measuring 13 x 8 mm.
- Left enlarged axillary lymph node with foci of micro calcifications inside and deeply hypoechoic echotexture, largest one measuring 9 x 5 mm.

CONCLUSION:-

- Left breast outer quadrant suspicious mass lesion with dimensions as described (for comparison with the previous study) (BIRADS: 6) on neo adjuvant CTH.
- Bilateral fibroadenosis with bilateral benign looking lesion represent fibroadenoma (BIRADS: 3).

MUCH OBLIGED
DR. SHAIMAA EL DIASTY, MD
PROF. DR. OMAR HUSSEIN
A.G.



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الست المصرية
صحة
مصر

MDT FORM

22/ 2 / 2023 GA

Name	مرفت عبد اللطيف سليم	MRN	٢٠٩٠٧١١١٢...٦٢
Age	٥٤	ID	١-٩ ٤-٥٣
Menopausal status	Post	Phone	١١-٢٢٢٢ ١٩٢
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity	DM - HTN	REASON OF MDT	new

Clinical Data

BX:

LT IDC grade III

ER:

018

PR:

018

HER 2:

+

Ki67:

30%

Sonomammography:

LT UOG 70X3.0X5.0 cm

with intraductal extension

fibrocystic

RT oval shaped lesions

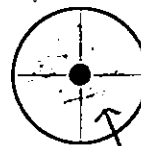
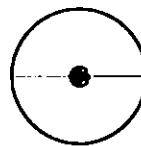
Mets Workup:

CT → free

new

R

L



TNM

these edges
extension of
the nodule

Recommendations:

new Sonomammogram at hand: RT UOG & UOG fibroadenoma
RT non specific ALN

LT T₁N₂M₀
or possibility of NACT then LT Mx

MDT OUTCOME & PLAN

start NACT (Taxol / Carbop / Hercept / Pegylate x6) is regular
clinical assessment. We consider the MDT @ Rptch
of RT breast fibroadenoma

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Visit Number: 61123501743

Client Name: نقابة المهندسين بنسبة - الناهرة

Patient Name: ميرفت عبد اللطيف سليم

Referred by Prof: -

Age / Sex: 64 year / Female

Exam Date: 22 Jan 2023

Patient ID: 2113488004

Printed: 1/22/2023 2:20:40 PM

US

Clinical data: Check-up.

Transthoracic Echocardiography

LVED	5.3	(3.5 - 5.7 cm)	LA	4.2	(1.9 - 4.0 cm)
LVES	2.7	(2.2 - 4.0 cm)	Ao	2.5	(2.0 - 3.7 cm)
SWT	1.0	(0.7 - 1.1 cm)	RV	2.4	(2.2 - 4.4 cm)
PWT	1.1	(0.7 - 1.1 cm)	MVA	5.2	(2.0 - 4.0 cm ²)
FS	49	(25 - 45 %)	EF	80	(54-82%)

Comments

- **Left ventricle:** Normal left ventricular internal dimensions with good overall global systolic function.
- **Diastolic dysfunction grade II (reversed E/A pattern).**
- There are no segmental wall motions abnormalities can be detected at rest study.
- **Left atrium:** Mildly dilated internal dimensions.
- **Mitral valve:** Thickened and non-calcified mitral valve leaflets with adequate excursion. Mild mitral regurgitation.
- **Aortic valve:** Trileaflet valve, normal leaflets thickness and excursions. No significant systolic gradient is present across the valve.
- **Right sided chambers:** Normal internal dimensions with preserved RV systolic function.
- **Tricuspid valve:** Normal leaflets thickness and excursion, trivial tricuspid regurgitation.
- No evidence of intracardiac masses or thrombi.
- **Pericardium:** Normal thickness, No pericardial effusion.
- **Intact Interatrial and interventricular septae** as seen by TTE.
- Normal thoracic aorta.



00 ش. عبد المنعم رياض - برج الأطباء - المهندسين

مشارف الميرفتي

محطة اللاون - فيصل

كابيتال بزنس بارك - BS - الدور الثالث - الشيخ زايد

٣٠ شارع الجزائر - المعادي

Echo

[DUPLICATE]

Visit Number: 61123501743
Patient Name: ميرفت عبد الصييف سليم
Age / Sex: 64 year / Female
Patient ID: 2113438004

Client Name: نقابة المهندسين بنسبة - القاهرة
Referred by Prof: -
Exam Date: 22 Jan 2023
Printed: 1/22/2023 2:20:40 PM

US

Conclusion:

- Degenerative heart disease with preserved systolic function.
- Impaired LV relaxation.
- Mild MR.
- Dilated LA.

Best Regards,
Dr. Amr Yehia, MSc



Visit Number:	61123501743	Client Name:	نقابة المهندسين بنسبة - القاهرة
Patient Name:	ميرفت عبد اللطيف سليم	Referred by Prof:	-
Age / Sex:	64 year / Female	Exam Date:	22 Jan 2023
Patient ID:	2113488C04	Printed:	1/23/2023 9:00:36 AM

MR

- The forementioned mass has intermediate T1 and T2 signals, with diffusion restriction on DWIs. On dynamic post contrast study, it showed heterogeneous enhancement, with type 3 time/intensity curve (moderate initial contrast uptake followed by early wash-out at 4.11 min).
- Diffuse skin thickening of the breast is seen (7 mm in thickness), with dilated subdermal lymphatics and diffuse interstitial breast edema.
- Pathological axillary level 1 and 2 lymph nodes are noted, few of which showing extracortical extension. Some of the lymph nodes are aggregated together, forming the largest nodal mass which measures around 3.5 x 1.5 cm in maximum dimensions.

RIGHT BREAST:

- Moderate background parenchymal enhancement of the breast (ACR c).
- A 2.0 x 1.5 cm, partially ill-defined, irregular mass is noted at 4-5 o'clock; exhibiting intermediate T1 and dark T2 signals. On dynamic post contrast study, the mass exhibited patchy nodular post contrast enhancement, with type 2 time/intensity curve (high initial contrast uptake, followed by increasing uptake then wash-out at 5.27 min).
- A 1.2 x 1.0 cm, benign-looking, well-defined mass is seen at 7 o'clock, exhibiting intermediate T1 and mostly bright T2 signals, demonstrating rather homogeneous post contrast enhancement and type 1 time/intensity curve; with minimal initial, followed by progressively increasing contrast uptake.
- An axillary lymph node is noted, with blunt hilum, measuring around 2.0 x 1.0 cm, for correlation with ultrasound.

Scan Result



مركز الأشعة - برج الأطباء - المهندسين

٢٠ شارع الجزائر

٦٧ شارع الميرغني

محطة التعاون - فيصل

كابيتال بيزنس بارك - BS - الدور الثالث - الشيخ زايد

٢٠ شارع الجزائر - المعادي

M.R.I

[DOCTORS.MR]

Visit Number: 61123501743

Patient Name: مبرقت عبد اللطيف سليم

Age / Sex: 64 year / Female

Patient ID: 2113488004

Client Name: نقابة المهندسين بلمبة - القاهرة

Referred by Prof: -

Exam Date: 22 Jan 2023

Printed: 1/23/2023 9:00:36 AM

MR

DYNAMIC MR MAMMOGRAPHY**INDICATIONS:** Left breast mass.**TECHNIQUE:**

- Axial T1, TRIM and T2 WIs.
- Sagittal T2 WIs.
- Five axial dynamic contrast-enhanced sequences, after bolus injection of 0.1 mmol/kg of Gadopentetate dimeglumine, using 1.6 mm slice thickness.
- MIP reconstruction.
- Diffusion-weighted images.
- Time/intensity curves created for enhancing lesions.

FINDINGS:**LEFT BREAST:**

- A large; around 7.0 x 3.0 x 5.0 cm (in anteroposterior, transverse and cranio-caudal dimensions, respectively), irregular mass is noted, occupying the UOQ of the breast, breast tail and 12 o'clock, extending to the retro-areolar region, with intraductal extension and nipple retraction.

Scan Result



Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : مرفت عبد النطيف سليم عبد المتعال

Analysis ID : 718314

Age : 63 Y : 6 M : 4 D

Gender : Female

Referred By Prof. Dr. : هشام الغزالي

Clinical Diagnosis : Left breast upper outer quadrant mass

Nature of Specimen : Block

Receiving Date : 2023-01-16

Patient ID : 549473

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain .

The sections were treated against:

Estrogen receptors

Progesterone receptors

Her 2

Ki67

Results :

Sections treated for Estrogen receptors revealed NEGATIVE nuclear staining.

Sections treated for Progesterone receptors revealed NEGATIVE nuclear staining.

Sections treated for HER2 revealed STRONG CONTINUOUS CELL MEMBRANE STAINING (CHICKEN WIRE) IN MORE THAN 10% OF TUMOR CELLS.

30 % of Tumour cells are POSITIVE for Ki67.

Conclusion :

Block, TUMOUR CELLS ARE

NEGATIVE FOR ESTROGEN RECEPTORS (SCORE 0 / 8).

NEGATIVE FOR PROGESTERONE RECEPTORS (SCORE 0 / 8).

POSITIVE FOR HER 2 PROTEIN OVEREXPRESSION, SCORE 3+.

THE PROLIFERATION INDEX OF KI67 IS 30 % OF TUMOUR CELLS.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia

Elia anis

This report is electronically signed , any abrasion or modification would render it void .



عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٩٠٩٣٥ - ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣

1/1

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
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Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : مرفت عبد اللطيف سليم عبد المتعال

Age : 63 Y : 6 M : 4 D

Referred By Prof. Dr. : هشام الغزالي

Clinical Diagnosis : Left breast upper outer quadrant mass.

Nature of Specimen : US guided tru cut biopsy.

Receiving Date : 2023-01-16

Analysis ID : 718313

Gender : Female

Patient ID : 549473

PATHOLOGY REPORT

Gross :

Multiple grayish tan tissue cores, the longest measured 1.2 cm long, totally submitted.

Microscopic :

Serial sections examined from the specimen received revealed cores of tumour tissue formed of irregular groups of malignant ductal cells, showing moderate nuclear pleomorphism and hyperchromasia.

There is moderate attempt at tubular formation.

The mitotic index is moderate. There is necrosis.

There is marked desmoplasia. There is peritumoural lymphocytic infiltrate around 10%.

There is intraductal component showing comedo pattern.

Diagnosis :

Left breast upper outer quadrant mass, US guided tru cut biopsy, INFLTRATING DUCT CARCINOMA, GRADE II-III.

- HORMONAL RECEPTORS ARE IN A SEPARATE REPORT.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia Anis

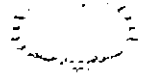
Elia Anis

This report is electronically signed , any abrasion or modification would render it void .

1/1

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٩٠٩٣٥ - ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣





Patient Name	: الأستاذ / مرفت عبداللطيف سليم	Request No.	: 13009106
Patient No.	: 25907111300062	Request Date	: 02/02/2023
Path No.	: 326/2023	Verification Date	: 11/02/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 63 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Known case of left breast cancer with right breast mass (BIRAD4).

Nature of the specimen: Tru-cut biopsy of right breast lesion. collected at 1/2/2023, 3:40 PM.

Gross Examination:

Four graigh white tissue cores, 1 cm longest core, totally submitted.

Microscopic examination:

Histopathologic examinations of prepared slides reveals breast tissue cores with ductectasia, focal papillomatosis, adenosis, fibroadenoma, and fibrosis.

Diagnosis:

Right breast mass; Tru-cut biopsy:

- Fibrocystic changes
- No malignancy

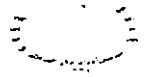
Prof.Dr. Iman Gouda
Dr.Heba mostafa

Verified by :
walaa ghoniem

Printing Date : 11/02/2023 14:50:34

Page 1 of 1

Printing By : .marwa mohamed youss



Patient Name	: الأستاذة / مرفت عبداللطيف سليم	Request No.	: 13009577
Patient No.	: 25907111300062	Request Date	: 04/02/2023
Path No.	: SR-98/23	Verification Date	: 11/02/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 63 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer

Nature of the specimen: Slide revision

Gross Examination:

One paraffin block labeled as (718313), 4 IPT slides, and 2 H&E stained slides were received.

Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): III (3+3+3)

Biomarkers:

ER: No nuclear reaction in tumor cells (Allred score 0/8)

PR: No nuclear reaction in tumor cells (Allred score 0/8)

Her-2: strong complete membrane reaction in >10% of tumor cells (score 3+)

Ki 67: 30 % nuclear reaction in tumor cells

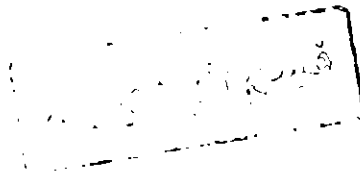
Diagnosis:

Left breast cancer; Slide revision:

- Invasive duct carcinoma, grade III
- ER: Negative (Allred score 0/8)
- PR: Negative (Allred score 0/8)
- HER2: Positive (Score 3+)
- Ki67: 30%

Prof.Dr. Iman Gouda
Dr.Heba Mostafa

Verified by :
walaa ghoniem



83123313

لجنة متابعة العلاج



الهيئة العامة للغذاء والدواء
Saudi Food & Drug Authority

RECEIVED
MDT

MDT FORM

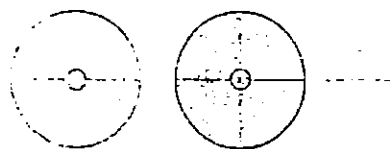
/ / 2023 GA

Name	مروة عبد الله سليم	MRN	2623/2023
Age	64	ID	25917111300062
Menopausal status	- past	Phone	01006333193
NRS SCORE		Marital status	married
BMI		Family history	-ve
CO morbidity	- DM. HTN	REASON OF MDT	past operative

Medical History

BX: rt breast:- post WLE → Fibroadenoma
L:- post NACT. FIRM →

ER:	PR:	HER 2:	KI67:
-----	-----	--------	-------



* left:- total tumor regression
in previously diagnosed carcinoma
- complete path response
- Free nipple areola & Free LNs.

Pathology Report

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : مرفت عبد اللطيف سليم عبد المتعال

Age : 63 Y : 6 M : 4 D

Referred By Prof. Dr. : هشام الغزالي

Clinical Diagnosis : Left breast upper outer quadrant mass.

Nature of Specimen : US guided tru cut biopsy.

Receiving Date : 2023-01-16

Analysis ID : 718313

Gender : Female

Patient ID : 549473

PATHOLOGY REPORT

Gross :

Multiple grayish tan tissue cores, the longest measured 1.2 cm long, totally submitted.

Microscopic :

Serial sections examined from the specimen received revealed cores of tumour tissue formed of irregular groups of malignant ductal cells, showing moderate nuclear pleomorphism and hyperchromasia.

There is moderate attempt at tubular formation.

The mitotic index is moderate. There is necrosis.

There is marked desmoplasia. There is peritumoural lymphocytic infiltrate around 10%.

There is intraductal component showing comedo pattern.

Diagnosis :

Left breast upper outer quadrant mass, US guided tru cut biopsy, INFILTRATING DUCT CARCINOMA, GRADE II-III.

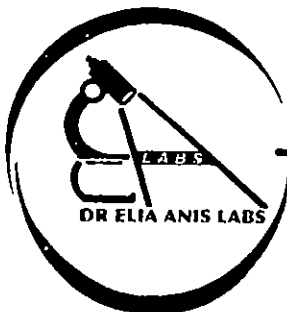
- HORMONAL RECEPTORS ARE IN A SEPARATE REPORT.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia

Elia Anis



This report is electronically signed , any abrasion or modification would render it void .

1/1

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٩٠٩٣٥ - ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : مرفت عبد اللطيف سليم عبد المتعال

Analysis ID : 718314

Age : 63 Y : 6 M : 4 D

Gender : Female

Referred By Prof. Dr. : هشام الغزالي

Clinical Diagnosis : Left breast upper outer quadrant mass

Nature of Specimen : Block

Receiving Date : 2023-01-16

Patient ID : 549473

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain .

The sections were treated against:

Estrogen receptors

Progesterone receptors

Her 2

Ki67

Results :

Sections treated for Estrogen receptors revealed NEGATIVE nuclear staining.

Sections treated for Progesterone receptors revealed NEGATIVE nuclear staining.

Sections treated for HER2 revealed STRONG CONTINUOUS CELL MEMBRANE

STAINING (CHICKEN WIRE) IN MORE THAN 10% OF TUMOR CELLS.

30 % of Tumour cells are POSITIVE for Ki67.

Conclusion :

Block, TUMOUR CELLS ARE

NEGATIVE FOR ESTROGEN RECEPTORS (SCORE 0 / 8).

NEGATIVE FOR PROGESTERONE RECEPTORS (SCORE 0 / 8).

POSITIVE FOR HER 2 PROTEIN OVEREXPRESSION, SCORE 3+.

THE PROLIFERATION INDEX OF KI67 IS 30 % OF TUMOUR CELLS.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia

Elia anis



This report is electronically signed , any abrasion or modification would render it void .

1/1

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٩٠٩٣٥ - ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣

Immunohistochemistry
[[DOCTYPE:IHC]]

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : مرفت عبد اللطيف سليم عبد المتعال

Age : 63 Y : 5 M : 1 D

Referred By Prof. Dr. :

Clinical Diagnosis : Left breast mass

Nature of Specimen : Block

Receiving Date : 2022-12-29

Analysis ID :  714266

Gender : Female

Patient ID : 549473

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view: HRP DAB as chromogen and hematoxylin as counter-stain .

The sections were treated against:

- Estrogen receptors
- Progesterone receptors.
- Ki67

Results :

Sections treated for Estrogen receptors revealed NEGATIVE nuclear staining.
5% of tumor cells show WEAK POSITIVE Staining for progesterone receptors.
25% of Tumour cells are POSITIVE for Ki67.

Conclusion :

Block, TUMOUR CELLS ARE
NEGATIVE FOR ESTROGEN RECEPTORS (SCORE 0 / 8)
POSITIVE FOR PROGESTERONE RECEPTORS (SCORE 3/ 8
THE PROLIFERATION INDEX OF KI67 IS 25% OF TUMOUR CELLS.
NB: HER2 NOT PERFORMED AS NO FRANK INVASON IN MATERIAL RECEIVED.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak



This report is electronically signed , any abrasion or modification would render it void .



Patient Name : الأستاذ / مرفت عبداللطيف سليم	Request No. : 13009577
Patient No. : 25907111300062	Request Date : 04/02/2023
Path No. : SR-98/23	Verification Date : 11/02/2023
Sex : Female	Financial Category : أورام دار السلام - نفقة الدولة
Age : 63 Y	Department : Out Patient
Referreing Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer

Nature of the specimen: Slide revision

Gross Examination:

One paraffin block labeled as (713313), 4 IPT slides, and 2 H&E stained slides were received.

Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): III (3+3+3)

Biomarkers:

ER: No nuclear reaction in tumor cells (Allred score 0/8)

PR: No nuclear reaction in tumor cells (Allred score 0/8)

Her-2: strong complete membrane reaction in >10% of tumor cells (score 3+)

Ki 67: 30 % nuclear reaction in tumor cells

Diagnosis:

Left breast cancer; Slide revision:

- Invasive duct carcinoma, grade III
- ER: Negative (Allred score 0/8)
- PR: Negative (Allred score 0/8)
- HER2: Positive (Score 3+)
- Ki67: 30%

Prof.Dr. Iman Gouda
Dr.Heba Mostafa

Verified by :

walaa ghoniem

Printing Date : 11/02/2023 14:50:17

Page 1 of 2

Printing By : marwa mohamed youss



Patient Name : الأستاذ / مرفت عبداللطيف سليم	Request No. : 13C09106
Patient No. : 25907111300062	Request Date : 02/02/2023
Path No. : 326/2023	Verification Date : 11/02/2023
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 63 Y	Department : Out Patient
Referring Dcc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Known case of left breast cancer with right breast mass (BIRAD4).

Nature of the specimen: Tru-cut biopsy of right breast lesion. collected at 1/2/2023, 3:40 PM.

Gross Examination:

Four graigh white tissue cores, 1 cm longest core, totally submitted.

Microscopic examination:

Histopathologic examinations of prepared slides reveals breast tissue cores with ductectasia, focal papillomatosis, adenosis, fibroadenoma, and fibrosis.

Diagnosis:

Right breast mass; Tru-cut biopsy:

- Fibrocystic changes
- No malignancy

Prof.Dr. Iman Gouda
Dr.Heba mostafa

Verified by :
walaa ghoniem



Patient Name: Mervat Abdel Latteef Selim Abd Al Motaal	Date : 11/09/2024
Exam : MAMMOGRAPHY BOTH	Age : 55 Year(s)
Medical ID: 359/2023	Pat. ID: 28723

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM.

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is heterogeneously dense which may obscure small lesions (ACR C),
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed right multiple well defined hypoechoic solid mass lesion with no internal vascularity seen within seen at 9o'clock and 3o'clock par areolar region measuring about 7x5mm and 22x14mm.
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Signature Dr. Nehal Hamdy

11/09/2024 2:15 PM

Patient Name: Mervat Abdal Latteef Se im Abd Al Motaal	Date : 15/02/2023
Exam : MAMMOGRAPHY BOTH	Age : 63 Year(s)
Medical ID: 359/2023	Pat. ID: 28723

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

.....

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B).
- left regional UOQ suspicious pieomorphic micro calcifications>>> pathologically proved carcinoma.
- Right UIQ well defined oval medium dense lesion
- No right suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- **Complementary ultrasound on targeted right breast** >>> right multiple well defined hypoechoic solid mass lesion with no internal vascularity seen within seen at 9o'clock and 3o'clock par areolar region measuring about 7x5mm and 22x14mm.
- Normal right skin thickness and nipple-areola complex.
- Right Nonspecific axillary LNs.

Opinion:

- Left pathologically proved carcinoma (B,RADS 6).
- right UOQ &UIQ benign looking lesion likely fibro adenoma for follow up after 6months. (BIRADS 3)

Yours truly

Signature Dr. Nehal Hamdy

15/02/2023 2:04 PM

جمهورية مصر العربية
بطاقة تحقيق الشخصية

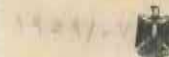


مرفق

عبداللطيف سليم عبدالمتعال

قطعه ٥١٦٠

المقطم - القاهرة



٢٥٩٠٧١١١٣٠٠٠٦٢

LC4837863



٢٠٢٥/٠٤ ٢٥٩.٧١١٣.٠٠٠٦٢

ربة منزل

ارملة

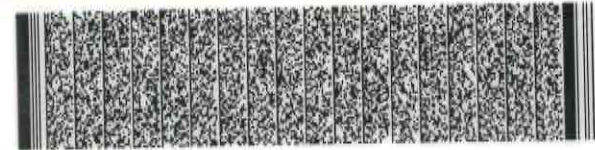
مسلمة

أنثى



البطاقة سارية حتى ٢٠٣٢/٠٤/١٥

١





Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : مرفت عبداللطيف سليمه عبدالمتعال Sex:

3- Medical number:

Male

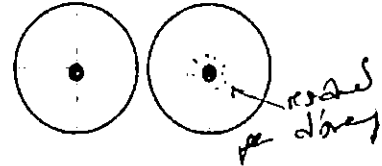
83123313

Old Med :

359/2023

2-ID number: 25907111330062

Patient Age : 64 Y



Family history

Phone

01006333193

BMI

REASON OF MDT

POST NACT

marital status

NRS Score

GROUP

A

Date of MDT

2023-07-19



Co morbidity

DM - H1A

Menopausal
status

POSTMENOPAUSAL

Clinical data

Breast Imaging :

LT UOQ 70*3.5*5.0 CM with intraductal extension
rt oval shaped lesion

Mets Workup :

c.t : free

BX:

ER:

0/8

PR:

0/8

Her 2 Neu :

3

SISH :

Ki67 :

30%

LT IDC GII

+VE

T

4D

N

1

M

0

Sonomammography

RT UOQ UIO FIBROADENOMA

LT UOQ 70*3.5*5.0 CM with intraductal extension

rt oval shaped lesion

PATIENT HISTORY

LABC-->HR-VE,,HER2 +VE

FINISHED TCX18 WK+HPX6

SM PENDING

FOR YOUR KIND CARE



ERM036727200

MDT OUTCOME & PLAN

for LT MRM & RT lumpectomy as per initial MDT decision

SIGNATURE

MEDICAL ONCOLOGY

د/ شيماء لاشين

RADIOLOGY

د/ نشور عبداللطيف

SURGERY

د/ محمد عبدالرحمن

PATHOLOGY

Psychiatric

Nutrition

MRS: Mervat Abd Al Latif Selim	Date: 2023/07/18
Dear. SIR.	ID: 928155

BILATERAL DIGITAL COMPUTED MAMMOGRAPHY AND SONOGRAPHY REVEALED:-

Known patient of left breast carcinoma on neo adjuvant CTH, the current study revealed:

The right breast:

- The right breast shows normal nipple, areola, skin and subcutaneous tissue thickness.
- It shows condensation of glandular tissue elements, with nodularity, suggestive of fibroadenosis.
- The right breast shows paraareolar inner quadrant 4 O'clock position lobulated opacity **by ultrasonography** it proved to be lobulated soft tissue lesion at 5 O'clock position measuring 17 x 10 mm, mostly represent fibroadenoma.
- The right breast shows few simple cysts measuring 3 x 2 mm.
- No suspicious breast masses are detected.
- No malignant micro calcifications are detected.
- No suspicious right axillary lymphadenopathy.

The left breast:

- The left breast shows mild diffuse skin and subcutaneous soft tissue thickening and edema.
- It shows condensation of glandular tissue elements, with nodularity, suggestive of fibroadenosis.
- It shows outer quadrant diffuse suspicious micro calcifications, **by ultrasonography** it proved to be area of underlying fibrosis and architectural distortion with suspicious mass lesion at 2 O'clock position now it measures 16 x 9 mm (for comparison with the previous study).

ا.د. أحمد كمال الدري
ا.د. محمد أبو الهدى درويش
ا.د. عمر حسين عمر
ا.د. وحيد حسين طنطاوى
أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

MRS: Mervat Abd Al Latif Selim	Date: 2023/07/18
Dear. SIR.	ID: 928155

- The left breast shows upper central retroareolar benign looking opacity with benign looking coarse calcifications inside measuring 13 x 8 mm.
- Left enlarged axillary lymph node with foci of micro calcifications inside and deeply hypoechoic echotexture, largest one measuring 9 x 5 mm.

CONCLUSION:-

- Left breast outer quadrant suspicious mass lesion with dimensions as described (for comparison with the previous study) (BIRADS: 6) on neo adjuvant CTH.
- Bilateral fibroadenosis with bilateral benign looking lesion represent fibroadenoma (BIRADS: 3).

MUCH OBLIGED
DR. SHAIMAA EL DIASTY, MD
PROF. DR. OMAR HUSSEIN
A.G.



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الست المصرية
صحة
مصر

MDT FORM

22/ 2 / 2023 GA

Name	مرفت عبد اللطيف سليم	MRN	٢٠٩٠٧١١١٢...٦٢
Age	٥٤	ID	١٠٩ ٤-٤٣
Menopausal status	Post	Phone	١١-٢٣٣٢ ١٩٢
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity	DM - HTN	REASON OF MDT	new

Clinical Data

BX:

LT IDC grade III

R

L

ER:

018

PR:

018

HER 2:

+

Ki67:

20%

Sonomammography:

LT UOG 70x3.0x5.0 cm with intraductal extension

fibrocystic changes - RT oval shaped lesions

Mets Workup:

CT - free

TNM

cT4d1

these edges
extension of
the nodule

Recommendations:

new Sonomammogram at hand: RT UOG & UOG fibroadenoma
RT non specific ALN

LT T4N2M0
possibility of NACT then LT mastectomy

MDT OUTCOME & PLAN

start NACT (Hormonal / Cyclophosphamide / Etoposide x 6) is regular
clinical assessment. We consider RT MDT @ R. prady
of RT breast fibroadenoma

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name: Mervat Abdelateef Sleem

65Y

F

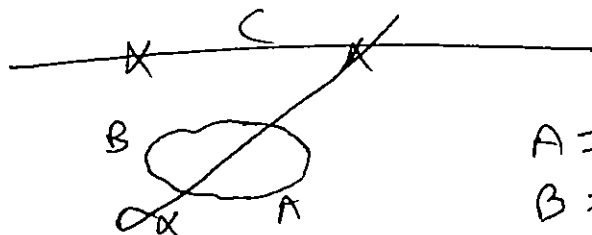
Date: 10/08/2023

Wire localization for right breast mass:

- Under local anesthesia, antiseptic condition and ultrasound guidance.
- Ultrasound guided wire localization was inserted into the right breast 3 o'clock lesion.
 - Whole distance of the wire till the tip: 2.5 cm
 - Perpendicular distance from skin to tip: 1.8 cm
 - Distance from perpendicular distance to skin entry: 1.8 cm.
- Skin mark over the lesion was done.

Best Regards,

**Dr. Ahmed M. Rozeka, Msc
Specialist Radiologist**



A = 2.5 cm.

B = 1.8 cm.

C = 1.8 cm.

Dr. Ahmed
Rozeka
10/8/2023

Visit Number: 61123501743

Client Name: نقابة المهندسين بنسبة - الناهرة

Patient Name: ميرفت عبد اللطيف سليم

Referred by Prof: -

Age / Sex: 64 year / Female

Exam Date: 22 Jan 2023

Patient ID: 2113488004

Printed: 1/22/2023 2:20:40 PM

US

Clinical data: Check-up.

Transthoracic Echocardiography

LVED	5.3	(3.5 - 5.7 cm)	LA	4.2	(1.9 - 4.0 cm)
LVES	2.7	(2.2 - 4.0 cm)	Ao	2.5	(2.0 - 3.7 cm)
SWT	1.0	(0.7 - 1.1 cm)	RV	2.4	(2.2 - 4.4 cm)
PWT	1.1	(0.7 - 1.1 cm)	MVA	5.2	(2.0 - 4.0 cm ²)
FS	49	(25 - 45 %)	EF	80	(54-82%)

Comments

- **Left ventricle:** Normal left ventricular internal dimensions with good overall global systolic function.
- **Diastolic dysfunction grade II (reversed E/A pattern).**
- There are no segmental wall motions abnormalities can be detected at rest study.
- **Left atrium:** Mildly dilated internal dimensions.
- **Mitral valve:** Thickened and non-calcified mitral valve leaflets with adequate excursion. Mild mitral regurgitation.
- **Aortic valve:** Trileaflet valve, normal leaflets thickness and excursions. No significant systolic gradient is present across the valve.
- **Right sided chambers:** Normal internal dimensions with preserved RV systolic function.
- **Tricuspid valve:** Normal leaflets thickness and excursion, trivial tricuspid regurgitation.
- No evidence of intracardiac masses or thrombi.
- **Pericardium:** Normal thickness, No pericardial effusion.
- **Intact Interatrial and interventricular septae** as seen by TTE.
- Normal thoracic aorta.



00 ش. عبد المنعم رياض - برج الأطباء - المهندسين

شارع الميرفتي

محطة اللاعون - فيصل

كابيتال بيننس بارك - BS - الدور الثالث - الشيخ زايد

٣٠ شارع الجزائر - المعادي

Visit Number: 61123501743
Patient Name: ميرفت عبد الصييف سليم
Age / Sex: 64 year / Female
Patient ID: 2113438004

Client Name: نقابة المهندسين بنسبة - القاهرة
Referred by Prof: -
Exam Date: 22 Jan 2023
Printed: 1/22/2023 2:20:40 PM

US

Conclusion:

- Degenerative heart disease with preserved systolic function.
- Impaired LV relaxation.
- Mild MR.
- Dilated LA.

Best Regards,
Dr. Amr Yehia, MSc



Visit Number:	61123501743	Client Name:	نقابة المهندسين بنسبة - القاهرة
Patient Name:	ميرفت عبد اللطيف سليم	Referred by Prof:	-
Age / Sex:	64 year / Female	Exam Date:	22 Jan 2023
Patient ID:	2113488C04	Printed:	1/23/2023 9:00:36 AM

MR

- The forementioned mass has intermediate T1 and T2 signals, with diffusion restriction on DWIs. On dynamic post contrast study, it showed heterogeneous enhancement, with type 3 time/intensity curve (moderate initial contrast uptake followed by early wash-out at 4.11 min).
- Diffuse skin thickening of the breast is seen (7 mm in thickness), with dilated subdermal lymphatics and diffuse interstitial breast edema.
- Pathological axillary level 1 and 2 lymph nodes are noted, few of which showing extracortical extension. Some of the lymph nodes are aggregated together, forming the largest nodal mass which measures around 3.5 x 1.5 cm in maximum dimensions.

RIGHT BREAST:

- Moderate background parenchymal enhancement of the breast (ACR c).
- A 2.0 x 1.5 cm, partially ill-defined, irregular mass is noted at 4-5 o'clock; exhibiting intermediate T1 and dark T2 signals. On dynamic post contrast study, the mass exhibited patchy nodular post contrast enhancement, with type 2 time/intensity curve (high initial contrast uptake, followed by increasing uptake then wash-out at 5.27 min).
- A 1.2 x 1.0 cm, benign-looking, well-defined mass is seen at 7 o'clock, exhibiting intermediate T1 and mostly bright T2 signals, demonstrating rather homogeneous post contrast enhancement and type 1 time/intensity curve; with minimal initial, followed by progressively increasing contrast uptake.
- An axillary lymph node is noted, with blunt hilum, measuring around 2.0 x 1.0 cm, for correlation with ultrasound.

Scan Result



مركز الأشعة - برج الأطباء - المهندسين

٢٠ شارع الجزائر - المعادي

٦٧ شارع الميرغني

محطة التعاون - فيصل

كابيتال بيزنس بارك - BS - الدور الثالث - الشيخ زايد

٢٠ شارع الجزائر - المعادي

Visit Number: 61123501743

Patient Name: مبرقت عبد اللطيف سليم

Age / Sex: 64 year / Female

Patient ID: 2113488004

Client Name: نقابة المهندسين بـمصر - القاهرة

Referred by Prof: -

Exam Date: 22 Jan 2023

Printed: 1/23/2023 9:00:36 AM

MR

DYNAMIC MR MAMMOGRAPHY**INDICATIONS:** Left breast mass.**TECHNIQUE:**

- Axial T1, TRIM and T2 WIs.
- Sagittal T2 WIs.
- Five axial dynamic contrast-enhanced sequences, after bolus injection of 0.1 mmol/kg of Gadopentetate dimeglumine, using 1.6 mm slice thickness.
- MIP reconstruction.
- Diffusion-weighted images.
- Time/intensity curves created for enhancing lesions.

FINDINGS:**LEFT BREAST:**

- A large; around 7.0 x 3.0 x 5.0 cm (in anteroposterior, transverse and cranio-caudal dimensions, respectively), irregular mass is noted, occupying the UOQ of the breast, breast tail and 12 o'clock, extending to the retro-areolar region, with intraductal extension and nipple retraction.

Scan Result



Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
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Dr.

Shady Elia Anis

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Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : مرفت عبد النطيف سليم عبد المتعال

Analysis ID : 718314

Age : 63 Y : 6 M : 4 D

Gender : Female

Referred By Prof. Dr. : هشام الغزالي

Clinical Diagnosis : Left breast upper outer quadrant mass

Nature of Specimen : Block

Receiving Date : 2023-01-16

Patient ID : 549473

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain .

The sections were treated against:

Estrogen receptors

Progesterone receptors

Her 2

Ki67

Results :

Sections treated for Estrogen receptors revealed NEGATIVE nuclear staining.

Sections treated for Progesterone receptors revealed NEGATIVE nuclear staining.

Sections treated for HER2 revealed STRONG CONTINUOUS CELL MEMBRANE STAINING (CHICKEN WIRE) IN MORE THAN 10% OF TUMOR CELLS.

30 % of Tumour cells are POSITIVE for Ki67.

Conclusion :

Block, TUMOUR CELLS ARE

NEGATIVE FOR ESTROGEN RECEPTORS (SCORE 0 / 8).

NEGATIVE FOR PROGESTERONE RECEPTORS (SCORE 0 / 8).

POSITIVE FOR HER 2 PROTEIN OVEREXPRESSION, SCORE 3+.

THE PROLIFERATION INDEX OF KI67 IS 30 % OF TUMOUR CELLS.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia

Elia anis

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عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٩٠٩٣٥ - ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣

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Member , United States Canadian Academy of Pathology

Patient Name : مرفت عبد اللطيف سليم عبد المتعال

Age : 63 Y : 6 M : 4 D

Referred By Prof. Dr. : هشام الغزالي

Clinical Diagnosis : Left breast upper outer quadrant mass.

Nature of Specimen : US guided tru cut biopsy.

Receiving Date : 2023-01-16

Analysis ID : 718313

Gender : Female

Patient ID : 549473

PATHOLOGY REPORT

Gross :

Multiple grayish tan tissue cores, the longest measured 1.2 cm long, totally submitted.

Microscopic :

Serial sections examined from the specimen received revealed cores of tumour tissue formed of irregular groups of malignant ductal cells, showing moderate nuclear pleomorphism and hyperchromasia.

There is moderate attempt at tubular formation.

The mitotic index is moderate. There is necrosis.

There is marked desmoplasia. There is peritumoural lymphocytic infiltrate around 10%.

There is intraductal component showing comedo pattern.

Diagnosis :

Left breast upper outer quadrant mass, US guided tru cut biopsy, INFILTRATING DUCT CARCINOMA, GRADE II-III.

- HORMONAL RECEPTORS ARE IN A SEPARATE REPORT.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia Anis

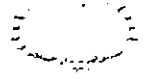
Elia Anis

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1/1

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٩٠٩٣٥ - ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣





Patient Name	: الأستاذة / مرفت عبداللطيف سليم	Request No.	: 13009106
Patient No.	: 25907111300062	Request Date	: 02/02/2023
Path No.	: 326/2023	Verification Date	: 11/02/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 63 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Known case of left breast cancer with right breast mass (BIRAD4).

Nature of the specimen: Tru-cut biopsy of right breast lesion. collected at 1/2/2023, 3:40 PM.

Gross Examination:

Four graigh white tissue cores, 1 cm longest core, totally submitted.

Microscopic examination:

Histopathologic examinations of prepared slides reveals breast tissue cores with ductectasia, focal papillomatosis, adenosis, fibroadenoma, and fibrosis.

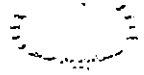
Diagnosis:

Right breast mass; Tru-cut biopsy:

- Fibrocystic changes
- No malignancy

Prof.Dr. Iman Gouda
Dr.Heba mostafa

Verified by :
walaa ghoniem



Patient Name	: الأستاذة / مرفت عبداللطيف سليم	Request No.	: 13009577
Patient No.	: 25907111300062	Request Date	: 04/02/2023
Path No.	: SR-98/23	Verification Date	: 11/02/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 63 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer

Nature of the specimen: Slide revision

Gross Examination:

One paraffin block labeled as (718313), 4 IPT slides, and 2 H&E stained slides were received.

Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): III (3+3+3)

Biomarkers:

ER: No nuclear reaction in tumor cells (Allred score 0/8)

PR: No nuclear reaction in tumor cells (Allred score 0/8)

Her-2: strong complete membrane reaction in >10% of tumor cells (score 3+)

Ki 67: 30 % nuclear reaction in tumor cells

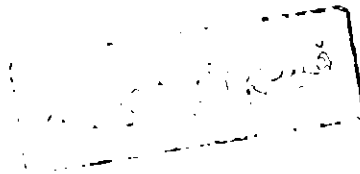
Diagnosis:

Left breast cancer; Slide revision:

- Invasive duct carcinoma, grade III
- ER: Negative (Allred score 0/8)
- PR: Negative (Allred score 0/8)
- HER2: Positive (Score 3+)
- Ki67: 30%

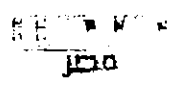
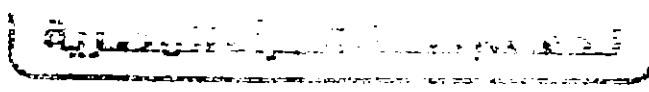
Prof.Dr. Iman Gouda
Dr.Heba Mostafa

Verified by :
walaa ghoniem



83123313

لجنة متابعة العلاج



MDT FORM

/

/ 2023

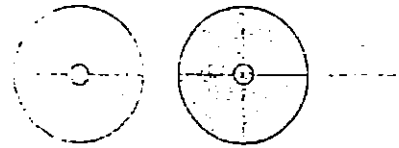
GA

Name	مروة عبد الله سليم	MRN	2623/2023
Age	64	ID	25917111300062
Menopausal status	- past	Phone	01006333193
NRS SCORE		Marital status	married
BMI		Family history	-ve
CO morbidity	- DM. HTN	REASON OF MDT	past operative

Medical History

BX: rt breast:- post WLE → Fibroadenoma
 Lt:- post NACT. FIRM →

ER:	PR:	HER 2:	KI67:
-----	-----	--------	-------



* left:- total tumor regression
 in previously diagnosed carcinoma
 - complete path response
 - Free nipple areola & Free LNs.

Physical Examination

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Signature:

Date: 12/12/2023

Dr.

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Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : مرفت عبد اللطيف سليم عبد المتعال

Age : 63 Y : 6 M : 4 D

Referred By Prof. Dr. : هشام الغزالي

Clinical Diagnosis : Left breast upper outer quadrant mass.

Nature of Specimen : US guided tru cut biopsy.

Receiving Date : 2023-01-16

Analysis ID : 718313

Gender : Female

Patient ID : 549473

PATHOLOGY REPORT

Gross :

Multiple grayish tan tissue cores, the longest measured 1.2 cm long, totally submitted.

Microscopic :

Serial sections examined from the specimen received revealed cores of tumour tissue formed of irregular groups of malignant ductal cells, showing moderate nuclear pleomorphism and hyperchromasia.

There is moderate attempt at tubular formation.

The mitotic index is moderate. There is necrosis.

There is marked desmoplasia. There is peritumoural lymphocytic infiltrate around 10%.

There is intraductal component showing comedo pattern.

Diagnosis :

Left breast upper outer quadrant mass, US guided tru cut biopsy, INFILTRATING DUCT CARCINOMA, GRADE II-III.

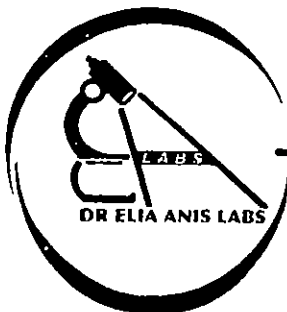
- HORMONAL RECEPTORS ARE IN A SEPARATE REPORT.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia Anis

Elia Anis



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Member , United States Canadian Academy of Pathology

Patient Name : مرفت عبد اللطيف سليم عبد المتعال

Analysis ID : 718314

Age : 63 Y : 6 M : 4 D

Gender : Female

Referred By Prof. Dr. : هشام الغزالي

Clinical Diagnosis : Left breast upper outer quadrant mass

Nature of Specimen : Block

Receiving Date : 2023-01-16

Patient ID : 549473

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain .

The sections were treated against:

Estrogen receptors

Progesterone receptors

Her 2

Ki67

Results :

Sections treated for Estrogen receptors revealed NEGATIVE nuclear staining.

Sections treated for Progesterone receptors revealed NEGATIVE nuclear staining.

Sections treated for HER2 revealed STRONG CONTINUOUS CELL MEMBRANE

STAINING (CHICKEN WIRE) IN MORE THAN 10% OF TUMOR CELLS.

30 % of Tumour cells are POSITIVE for Ki67.

Conclusion :

Block, TUMOUR CELLS ARE

NEGATIVE FOR ESTROGEN RECEPTORS (SCORE 0 / 8).

NEGATIVE FOR PROGESTERONE RECEPTORS (SCORE 0 / 8).

POSITIVE FOR HER 2 PROTEIN OVEREXPRESSION, SCORE 3+.

THE PROLIFERATION INDEX OF KI67 IS 30 % OF TUMOUR CELLS.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia Anis

Elia Anis



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Dr.

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Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : مرفت عبد اللطيف سليم عبد المتعال

Age : 63 Y : 5 M : 1 D

Referred By Prof. Dr. :

Clinical Diagnosis : Left breast mass

Nature of Specimen : Block

Receiving Date : 2022-12-29

Analysis ID :  714266

Gender : Female

Patient ID : 549473

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view: HRP DAB as chromogen and hematoxylin as counter-stain .

The sections were treated against:

- Estrogen receptors
- Progesterone receptors.
- Ki67

Results :

Sections treated for Estrogen receptors revealed NEGATIVE nuclear staining.
5% of tumor cells show WEAK POSITIVE Staining for progesterone receptors.
25% of Tumour cells are POSITIVE for Ki67.

Conclusion :

Block, TUMOUR CELLS ARE
NEGATIVE FOR ESTROGEN RECEPTORS (SCORE 0 / 8)
POSITIVE FOR PROGESTERONE RECEPTORS (SCORE 3/ 8
THE PROLIFERATION INDEX OF KI67 IS 25% OF TUMOUR CELLS.
NB: HER2 NOT PERFORMED AS NO FRANK INVASON IN MATERIAL RECEIVED.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak



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Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name : الأستاذ / مرفت عبداللطيف سليم	Request No. : 13009577
Patient No. : 25907111300062	Request Date : 04/02/2023
Path No. : SR-98/23	Verification Date : 11/02/2023
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 63 Y	Department : Out Patient
Referreing Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer

Nature of the specimen: Slide revision

Gross Examination:

One paraffin block labeled as (713313), 4 IPT slides, and 2 H&E stained slides were received.

Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): III (3+3+3)

Biomarkers:

ER: No nuclear reaction in tumor cells (Allred score 0/8)

PR: No nuclear reaction in tumor cells (Allred score 0/8)

Her-2: strong complete membrane reaction in >10% of tumor cells (score 3+)

Ki 67: 30 % nuclear reaction in tumor cells

Diagnosis:

Left breast cancer; Slide revision:

- Invasive duct carcinoma, grade III
- ER: Negative (Allred score 0/8)
- PR: Negative (Allred score 0/8)
- HER2: Positive (Score 3+)
- Ki67: 30%

Prof.Dr. Iman Gouda
Dr.Heba Mostafa

Verified by :
walaa ghoniem

Printing Date : 11/02/2023 14:50:17

Page 1 of 2

Printing By : marwa mohamed youss



Patient Name : الأستاذ / مرفت عبداللطيف سليم	Request No. : 13C09106
Patient No. : 25907111300062	Request Date : 02/02/2023
Path No. : 326/2023	Verification Date : 11/02/2023
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 63 Y	Department : Out Patient
Referring Dcc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Known case of left breast cancer with right breast mass (BIRAD4).

Nature of the specimen: Tru-cut biopsy of right breast lesion. collected at 1/2/2023, 3:40 PM.

Gross Examination:

Four graigh white tissue cores, 1 cm longest core, totally submitted.

Microscopic examination:

Histopathologic examinations of prepared slides reveals breast tissue cores with ductectasia, focal papillomatosis, adenosis, fibroadenoma, and fibrosis.

Diagnosis:

Right breast mass; Tru-cut biopsy:

- Fibrocystic changes
- No malignancy

Prof.Dr. Iman Gouda
Dr.Heba mostafa

Verified by :
walaa ghoniem



End of Treatment Form

Patient Name: مبركة عبد الطيف File Number: ٢٠٢٣/٢١١٨

Age: ٦٤

Gender: ♀

Diagnosis: left breast cancer

Treatment Received: PORT

Technique: 3D

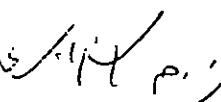
Dosage: 40 Gy / 15 fx / 3 weeks + bolus last 7 fx

Site: H CW + RNI

Start Date: 30-9-2023

End Date: 18-10-2023

Overall Treatment Time: 15 days / 3 weeks

Resident: 

Consultant:

Date:



Patient Name: Mervat Abdel Latteef Selim Abd Al Motaal	Date : 11/09/2024
Exam : MAMMOGRAPHY BOTH	Age : 55 Year(s)
Medical ID: 359/2023	Pat. ID: 28723

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM.

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is heterogeneously dense which may obscure small lesions (ACR C),
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed right multiple well defined hypoechoic solid mass lesion with no internal vascularity seen within seen at 9o'clock and 3o'clock par areolar region measuring about 7x5mm and 22x14mm.
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Signature Dr. Nehal Hamdy

11/09/2024 2:15 PM

Patient Name: Mervat Abdal Latteef Se im Abd Al Motaal	Date : 15/02/2023
Exam : MAMMOGRAPHY BOTH	Age : 63 Year(s)
Medical ID: 359/2023	Pat. ID: 28723

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

.....

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B).
- left regional UOQ suspicious pieomorphic micro calcifications>>> pathologically proved carcinoma.
- Right UIQ well defined oval medium dense lesion
- No right suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- **Complementary ultrasound on targeted right breast** >>> right multiple well defined hypoechoic solid mass lesion with no internal vascularity seen within seen at 9o'clock and 3o'clock par areolar region measuring about 7x5mm and 22x14mm.
- Normal right skin thickness and nipple-areola complex.
- Right Nonspecific axillary LNs.

Opinion:

- Left pathologically proved carcinoma (B,RADS 6).
- right UOQ & UIQ benign looking lesion likely fibro adenoma for follow up after 6months. (BIRADS 3)

Yours truly

Signature Dr. Nehal Hamdy

15/02/2023 2:04 PM



Patient Name : الأستاذ / مرفت عبداللطيف سليم	Request No. : 13007630
Patient No. : 25907111300062	Request Date : 12/08/2023
Path No. : 2623/23	Verification Date : 21/08/2023
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 64 Y	Department : Out Patient
Referreing Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Bilateral breast cancer, post NACT.

Nature Specimen: Right WLE and Left modified radical mastectomy specimen.

Gross Examination:

Right breast :

* Right WLE specimen measured 7x6cm, with covering skin ellipse measured 3x1.5 cm , cut section revealed fairly circumscribed grayish white rubbery lobulated mass , 0.5 cm from deep margin .

Left breast :

* Left MRM measured 26x22x6 cm with covering skin ellipse measured 19x10 cm.

* Tumor extent: A greyish white firm tumor bed site.

* Size: 10x5x4 cm.

* Site : Central .

* Skin involvement: Nipple & areola were unremarkable grossly.

* Distance from margins: 2cm away from deep margin.

* Axilla measured 7x7 cm entagling 11 L.Ns, measured up to 1.2cm in diameter.

Microscopic Examination:

Right breast :

Right breast, WLE :

-Histopathological examination reveals a benign growth made up of proliferating small patent mammary ducts. Some of the ducts are compressed and closed, other are patent. The ducts are lined by inner columnar and outer myoepithelial cells. The strcma in between is formed of loose fibrous connective tissue, associated with diffuse stromal fibrosis, focal adenosis , irregular and cystically dilated ducts, and ducts showing apocrine metaplasia. The ducts are lined by inner columnar and outer myoepithelial cells.

Verified by :

Dr.Marwa mahmoud



Patient Name	: الأستاذة / مرفت عبداللطيف سليم	Request No.	: 13007630
Patient No.	: 25907111300062	Request Date	: 12/08/2023
Path No.	: 2623/23	Verification Date	: 21/08/2023
Sex	: Female	Financial Category	: نفقة الدونة - اورام دار السلام
Age	: 64 Y	Department	: Out Patient
Referreing Doc :		Registered by	: marwa mohamed youss

Pathology Report

Left breast :

- Total tumor regression
- Complete pathological response .
- There is hyalinosis , calcifications , and histiocytic aggregats , rest of the breast showed fibrocystic changes and fibroadenosis .
- * Skin involvement: Free Nipple & areola.
- * Lymph node involvement: Free (0/9) , one lymph node showed microcalcification and hyalinosis .
- * Resection Margins: Free deep margin .

Diagnosis:

- *1- Right breast , WLE;
Fibroadenoma and fibrocystic changes, No malignancy.

*2- Left breast cancer, post NACT/MRM specimen:✓

- Total tumor regression in a previously diagnosed carcinoma.
- Complete pathological response.
- Free Deep margin .
- Free nipple and areola .
- Free LNs (0/9) .
- Stage: ypT0, ypN0.

ج
م

Signature

Prof. Lobna Omar

Dr. Marwa Mahmoud

Verified by :

Dr.Marwa mahmoud



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : مرفت عبداللطيف سليم عبدالمنعم Sex:

3- Medical number:

Male

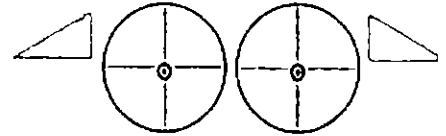
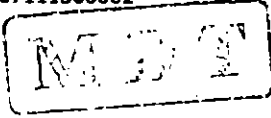
83123313

Old Med :

2623/2023

2-ID number: 25907111300062

Patient Age : 64 Y



Family history

-VE

Phone

01006333193

BMI

REASON OF MDT

POST OPERATIVE

marital status

MARRID

NRS Score

GROUP

A

Date of MDT

2023-08-26

Co morbidity

DM

HTN

Menopausal
status

POSTMENOPAUSAL

Clinical data

Breast Imaging :

Mets Workup :

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Sonomammography

PATIENT HISTORY

RT BREAST POST WLE --> FIBROADENOMA

LT POST NACTH TCHP MRM

LT TOTAL TUMOR REGRESSION IN A PREVIOUSLY DIAGNOSED CARCINOMA,,CPR,,FREE NIPPLE AND AREOLA,,FREE LNS

MDT OUTCOME & PLAN

FOR ADJUVANT HP FOR 1 YEAR& PORT.

SIGNATURE

MEDICAL ONCOLOGY

د/ هاجر القزوي

RADIOLOGY

د/ هاجر نور

SURGERY

PATHOLOGY

Psychiatric

Nutrition

9. 22

[illegible]

○ عبادة

○ مستشفى اخري ○ خروج حسب الطلب ○ منزل

الاسم :
التوقيع :
التاريخ : ٢٠ / /

alliance 25
Cibola 24

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83123313
مصرف عبد الحفيظ، بطريق عبد السلام
دور
بغداد
31/01/2023
2590711130006212
HSE 112

أقر أنا الموظف العام عبدالله بن محمد بن عبد الله بصفتي (صلة القرابة للمرحوم) الأب.....

ممتلكات المريض الثمينة :

- ### واجبات المريض

- ## حقوق المريض

- ### توقيع الموظف المسئول:

24-2-20

توقيع المقر في محمد بن عبد الله بن أبي القوام

التاريخ ٩ / ٧ / ١٤٤٥ هـ الوقت ١٢ : ١١ ص

كود الممثل	كود المراجع	كود الطبيب
١٤٣٨٩	٦٢٨٩	٢١٢٥٠

توصية المجلس الطبي المتخصص

رقم ٢٨٠١١٢٥٣ لسنة ٢٠٢٣
الرقم القومي ٢٥٩٠٧١١١٣٠٠٠٦٢

الإدارة العامة للمجالس الطبية المتخصصة

- بعد الاطلاع علي تقرير الكشف الطبي والابحاث الخاصه بالمواطن المذكور ادناه
- ووفقا لما جاء بقرار وزير الصحة رقم ٤٠٨ لسنة ٢٠٠٩
- وعلي توصية المجلس الطبي المتخصص رقم ٢٧٩٥١٠٨٥ لسنة ٢٧/٠٧/٢٠٢٣
- وعلي تقرير اللجنة الثلاثية الصادر من المستشفى بتاريخ ١٩/٠٧/٢٠٢٣
- اوصي المجلس الطبي المتخصص بجلسة ٠١/٠٨/٢٠٢٣

بالموافقة علي استكمال علاج المواطن / مرفت عبد النطيف سليم عبد المتعال من سرطان بالثدي (مبادرة) الصادر في شأنه التوصية رقم ٢٧٩٥١٠٨٥ بمستشفى مركز اورام دار السلام بقسم الاورام لمدة ٧ يوم خارجي وداخلي تدخل جراحي لاستئصال كامل /تحفظي للثدي +تفريغ الغدد الليمفاوية و/ او الغدد الحارسة (كود ١١٩١٤).

وفقا لما جاء باسعار بروتوكولات العلاج علي نفقة الدولة وطبقا لكود الحالة المرضية

تحريرا في ٠١/٠٨/٢٠٢٣

تعتبر التوصية نافذه من تاريخ تحرير اللجنة الثلاثيه
قيمة القرار مخصصة للعلاج ولا يجوز المطالبة بأي نفقات أخرى سواء للإنتقال أو أي إجراء خارج منطوق القرار

مدير عام

الإدارة العامة للمجالس الطبية المتخصصة

د. محمد حسن زيدان

مراجعة طبية

د/ حيدر ابراهيم

مراجعة ادارية

فلطمة فرجاتي

مركز اورام دار السلام
طبع في : ٠٩/٠٨/٢٠٢٣ ٥٢٢٠٢٠٩٨



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام " هرمل "
ادارة الجودة

انن دخول

اسم المريض : محمد حسن عبد الرحمن سالم

الرقم الطبي : 352/2023

السن : 64

التشخيص : lt : rt breast mass

الإجراء المطلوب : Far R.t lumpectomy

and
lett TLR

الطبيب المعالج : د. محمد

توقيع الطبيب

د. محمد

Visit Number:	61123501743	Client Name:	نقابة المهندسين بنسبة - القاهرة
Patient Name:	مروفت عبد اللطيف سليم	Referred by Prof:	-
Age / Sex:	64 year / Female	Exam Date:	22 Jan 2023
Patient ID:	2113488004	Printed:	1/23/2023 9:00:36 AM

MR

DYNAMIC MR MAMMOGRAPHY

INDICATIONS: Left breast mass.

TECHNIQUE:

- Axial T1, TRIM and T2 WIs.
- Sagittal T2 WIs.
- Five axial dynamic contrast-enhanced sequences, after bolus injection of 0.1 mmol/kg of Gadopentetate dimeglumine, using 1.6 mm slice thickness.
- MIP reconstruction.
- Diffusion-weighted images.
- Time/intensity curves created for enhancing lesions.

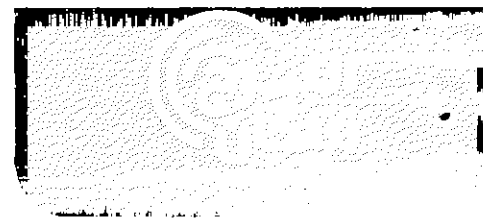
FINDINGS:

LEFT BREAST:

- A large; around 7.0 x 3.0 x 5.0 cm (in anteroposterior, transverse and craniocaudal dimensions, respectively), irregular mass is noted, occupying the UOQ of the breast, breast tail and 10 o'clock, extending to the retro-areolar region, with intraductal extension and nipple retraction.

Scan Result





Visit Number:	61123501743	Client Name:	نقابة المهندسين بنسبة - القاهرة
Patient Name:	ميرفت عبد اللطيف سليم	Referred by Prof:	-
Age / Sex:	64 year / Female	Exam Date:	22 Jan 2023
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MR

- The forementioned mass has intermediate T1 and T2 signals, with diffusion restriction on DWIs. On dynamic post contrast study, it showed heterogeneous enhancement, with type 3 time/intensity curve (moderate initial contrast uptake followed by early wash-out at 4.11 min).
- Diffuse skin thickening of the breast is seen (7 mm in thickness), with dilated subdermal lymphatics and diffuse interstitial breast edema.
- Pathological axillary level 1 and 2 lymph nodes are noted, few of which showing extracortical extension. Some of the lymph nodes are aggregated together, forming the largest nodal mass which measures around 3.5 x 1.5 cm in maximum dimensions.

RIGHT BREAST:

- Moderate background parenchymal enhancement of the breast (ACR c).
- A 2.0 x 1.5 cm, partially ill-defined, irregular mass is noted at 4-5 o'clock; exhibiting intermediate T1 and dark T2 signals. On dynamic post contrast study, the mass exhibited patchy nodular post contrast enhancement, with type 2 time/intensity curve (high initial contrast uptake, followed by increasing uptake then wash-out at 5.27 min).
- A 1.2 x 1.0 cm, benign-looking, well-defined mass is seen at 7 o'clock, exhibiting intermediate T1 and mostly bright T2 signals, demonstrating rather homogeneous post contrast enhancement and type 1 time/intensity curve; with minimal initial, followed by progressively increasing contrast uptake.
- An axillary lymph node is noted, with blunt hilum, measuring around 2.0 x 1.0 cm, for correlation with ultrasound.

Scan Result



- برج الأطباء - المهندسين

- فيصل

إيزنس بارك - BS - الدور الثالث - الشيخ زايد

- المعادي



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Visit Number:	61123501743	Client Name:	نقابة المهندسين بنمية - القاهرة
Patient Name:	ميرفت عبد الطيف سليم	Referred by Prof:	-
Age / Sex	64 yea / Female	Exam Date:	22 Jan 2023
Patient ID	2113483004	Printed:	1/23/2023 9:00:36 AM

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OPINION:

- Large left breast mass, with pathological axillary lymphadenopathy, as described (BIRADS V).
- Right breast mass and indeterminate axillary lymph node (BIRADS IV).
- Further assessment by biopsy is recommended.

Thank you for your reference,
Prof. Dr. Fatma Awad,
Cairo University

Scan Result



00. ش. عبد المنعم رياض - برج الأطباء - المهندسين

01. شارع الميرغني

محطة التعاون - فيصل

كابتال بيزنس بارك - BS - الدور الثالث - الشيخ زايد

3. شارع الجزائر - المعادي

Name : Mrs. Mervat Abd El Latif Selim MSALME

Date : 25/12/2022

Code : R2823746

3d full field DIGITAL MAMMOGRAPHY AND TOMOSYNTHESIS
EXAMINATION OF BOTH BREASTS:

CLINICAL SUMMARY: 62 yrs old female patient , left breast lump with retracted nipple

IMPRESSION:

RIGHT BREAST (BIRADS 3): multiple probably benign lesions likely fibroadenomata

LEFT BREAST (BIRADS 5): advanced multicentric lesions with highly suspicious micro-calcifications and ipsilateral pathological LNDS level I&II

RECOMMENDATION:

Biopsy and CESM correlation

TECHNIQUE:

Cranio-caudal & Mediolateral Oblique views

High resolution ultrasound

Tomosynthesis in 0.5mm slice thickness

DETAILED FINDINGS:

Breast composition:

The breasts show scattered fibroglandular tissues (ACR b)

Right Breast:

- Multiple well defined oval shaped hyper-dense lesions and scattered macro-calcifications .
- Additional ultrasound revealed normal parenchyma, and shows multiple well defined hypoechoic lesions the largest retro-areolar macro-lobulated measured about 18x12mm
- Normal skin thickness and contour.
- Normal retro-areolar ductal system.
- Normal nipple/areola complex.
- No abnormal axillary lymph nodes.

Name : Mrs. Mervat Abd El Latif Sellm MSALME

Date : 25/12/2022

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Left Breast:

- Most of the breast is seen infiltrated by suspicious polymorphic micro-calcifications as well as retracted nipple and retroareolar architecture distortion and skin thickness
- Upper central well defined oval shaped lesion with internal macro-calcifications
- Additional ultrasound revealed from 2 o clock to 6 o clock zone A&B & C and 6 o clock to 9 o clock zone A & B altered parenchyma with internal suspicious calcifications associated with skin thickness reached about 5mm and subcutaneous edema
- multiple ill defined hypoechoic speculated lesions detected the largest one at 3 o clock deeply seated on the muscle measured about 3.1x2.3cm away from the skin by 3 cm
- retroareolar area of altered echogenicity reached to nipple with architecture distortion .
- at 12 o clock shows well defined oval shaped lesion with internal macrocalcifications measured about 14x9mm
- multiple pathological lymphnodes more than 3 at level one two of them show internal suspicious calcifications the largest measured about 15mm with cortical thickness reached about 6mm and one at level II measured about 9mm .

Sincerely,
Dr. Marwa Abdelhakim ,msc

*For any inquiries regarding the findings in the report please contact us on
radreport@cairoscan.com.eg*

Name : Mrs. Mervat Abd El Latif Selim MSALME

Date : 25/12/2022

Code : R2823746

Definition of Breast Imaging Reporting and Data System

(BI-RADS):

Category			
0	Needs further imaging study		
1	Negative (there is nothing to comment on)		
2	Benign Finding		
3	Probably Benign Finding	< 2% malignant risk	short term follow up is recommended
4	Suspicious Abnormality	2- 95% malignant risk	Blopsy should be considered
	4a	Small risk	
	4b	Moderate risk	
	4c	Substantial risk	
5	Highly suggestive of Malignancy	> 95% malignant risk	Appropriate Action should be taken
6	Known Blopsy	Proven Malignancy	

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : مرفت عبد اللطيف سليم عبد المتعال

Age : 63 Y : 5 M : 1 D

Referred By Prof. Dr. :

Clinical Diagnosis : Left breast mass

Nature of Specimen : US guided biopsy

Receiving Date : 2022-12-29

Analysis ID :  714265

Gender : Female

Patient ID : 549473

PATHOLOGY REPORT

Gross :

Multiple greyish white tissue cores, the longest measured 0.7 cm in dimension, totally submitted

Microscopic :

Serial sections examined from the specimen received cores of breast tissue with ductal carcinoma insitu formed of dilated ducts with moderately pleomorphic malignant lining showing solid and cribriform pattern.

There are foci of microcalcification . No frank invasion in material received.

There is peritumoural lymphocytic infiltrate around 20%.

Diagnosis :

Left breast mass, US guided biopsy, DUCTAL CARCINOMA INSITU (SOLID AND CRIBRIFORM), WITH FOCI OF MICROCALCIFICATION, NO FRANK INVASION IN MATERIAL RECEIVED.

- IF INVAION IS SUSPECTED RADIOLOGICALLY MORE TISSUE BIOPSY IS MANDATORY.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak



This report is electronically signed , any abrasion or modification would render it void .

Elia (1)

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

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Prof. Dr. Elia Anis Ishak

Shady Elia

Elia anis

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Visit Number: 61123501743
Patient Name: ميرفت عبد اللطيف سليم
Age / Sex: 64 year / Female
Patient ID: 2113488004

Client Name: نقابة المهندسين بنسبة - القاهرة
Referred by Prof: -
Exam Date: 22 Jan 2023
Printed: 1/23/2023 9:00:36 AM

MR

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Cairo University.

Scan Result



رياض - برج الأطباء - المهندسين

الميرغني

الون - فيصل

الاراك - BS - الدور الثالث - الشيخ زايد

الجزائر - المعادي



Imaging report



كايرو سكان
للأشعة والتحاليل
CAIRO SCAN
Radiology & LABS

Name : Mrs. Mervat Abd El Latif Sellm
MSALME

Date : 12/01/2023

PET CT GE Discovery IQ

Prof. Hany Wealem

Code : R2823746

- Dose/Radiopharmaceutical: F-18 Fluorodeoxyglucose (FDG) was administered via a peripheral upper limb vein.
- Imaging was performed on an integrated PET/CT scanner approximately 60 minutes after injection of the radiopharmaceutical.
- A low-dose non-contrast CT images for attenuation correction and an anatomic localisation were obtained followed by PET images from the vertex of the skull to the thighs.
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- The Standardised Uptake Value maximum variant (SUV max) was calculated within regions of interest whenever needed

Breasts and axillae:

- A large FDG avid lobulated enhanced soft tissue mass is seen involving the LOQ of the left breast measuring about 3.3 x 2.4 cm in its axial dimensions with SUVmax 9.9 with smaller non FDG avid enhanced nodules within the fibroglandular breast parenchyma.
- Multiple pathologically enlarged FDG avid left axillary LNs with the largest measuring about 3 x 1.2 cm and most active eliciting SUVmax3.5.
- FDG avid enlarged left subpectoral LN measuring about 12mm in diameter eliciting SUVmax4.6.

Head and Neck:-

- The brain appears normal on CT, with no abnormal focal metabolic activity detected.
- No pathologically enlarged or metabolically active cervical lymphadenopathy.



Imaging report



كايرو سكين
للشعة والتحاليل
CAIRO SCAN
Radiology & LABS

Name : Mrs. Mervat Abd El Latif Sellm
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PET CT GE Discovery IQ

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Code : R2823746

Whole body (FDG) PET/CT showing:

- **Metabolically active neoplastic left breast mass with metabolically active metastatic left axillary&pectoral nodal deposits**
- **No gross hypermetabolic lesions detected at the rest of the scanned body.**

Best Regards

Nuclear medicine consultant

Prof.Dr. Maha Abd Elkareem, MD

A. professor of Nuclear Medicine,
Kasr al Ainy, Cairo University

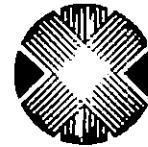
Radiology Consultant

Dr. Ahmed Mahmoud, MD

*For any inquiries regarding the findings in the report please contact us on
radreport@cairoscan.com.eg*



Imaging report



كايرو سكان
للشعة والتحاليل
CAIRO SCAN
Radiology & LABS

Name : Mrs. Mervat Abd El Latif Selim
MSALME

Date : 12/01/2023

PET CT GE Discovery IQ

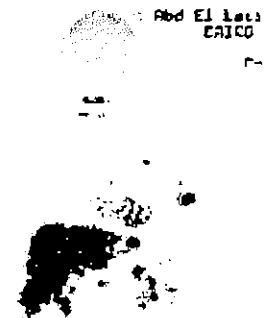
Prof. Hany Wealem

Code : R2823746

- Physiologic radiotracer uptake is demonstrated at the oropharynx, larynx, thyroid and salivary glands.
- Normal FDG uptake pattern is seen at the soft tissues of the neck.

Chest:

- Clear both lung fields with no metabolically active pulmonary nodules or parenchymal abnormalities demonstrated.
- No pleural or pericardial collections.
- No metabolically active or pathologically enlarged supra-clavicular, hilar or mediastinal lymph nodes could be detected.
- Normal tracer biodistribution is noted at the myocardium and great vessels.



Abdomen and Pelvis:

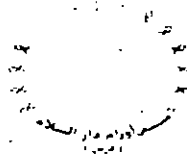
- Liver is mildly enlarged (20 cm span) showing homogenous texture and smooth outline. No hepatic focal lesions or areas of abnormal enhancement or metabolic activity. No biliary radicles dilatations could be noted.
- Normal CT appearance of the spleen, pancreas, adrenals, both kidneys, aorta and IVC as well as pelvic organs
- Normal FDG activity is seen at the gastrointestinal and genitourinary systems, with no striking focal hypermetabolic lesions detected.
- No pathologically enlarged or metabolically abdominal or pelvic lymph nodes.
- No ascites.



Musculo-skeletal:

- Non FDG avid bony island at right femoral greater trochanter
- FDG biodistribution is rather physiologic, with no significant blastic or lytic skeletal lesions could be seen on CT to account for metastatic disease to bone.





Patient Name: Mervat Abdel Latteef Selim Abd Al Motaal	Date : 15/02/2023
Exam : MAMMOGRAPHY BOTH	Age : 63 Year(s)
Medical ID: 359/2023	Pat. ID: 28723

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

.....

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B).
- left regional UOQ suspicious pleomorphic micro calcifications>>> pathologically proved carcinoma.
- Right UIQ well defined oval medium dense lesion
- No right suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- Complementary ultrasound on targeted right breast >>>> right multiple well defined hypoechoic solid mass lesion with no internal vascularity seen within seen at 9o'clock and 3o'clock par areolar region measuring about 7x5mm and 22x14mm.
- Normal right skin thickness and nipple-areola complex.
- Right Nonspecific axillary LNs.

Opinion:

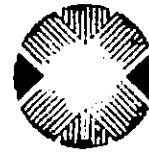
- Left pathologically proved carcinoma (BIRADS 6).
- right UOQ & UIQ benign looking lesion likely fibro adenoma for follow up after 6months. (BIRADS 3)

Yours truly

Signature Dr. Nehal Hamdy

15/02/2023 2:04 PM

Imaging report



كايرو سكان
CAIRO SCAN
PET/CT & SPECT

Name : Mrs. Mervat Abd El Latif Selim
MSALME

Date : 12/01/2023
PET CT GE Discovery IQ

Prof. Hany Wealem

Code : R2823746

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Imaging report



كايرو سكين
CAIRO SCAN
Radiology & AI

Name : Mrs. Mervat Abd El Latif Selim
MSALME
Date : 12/01/2023
PET CT GE Discovery IQ

Prof. Hany Wealem

Code : R2823746

CAIRO SCAN

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Imaging report



كايرو سكين
CAIRO SCAN
Radiology A1132

Name : Mrs. Mervat Abd El Latif Selim
MSALME

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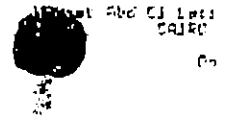
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Musculo-skeletal:

- Non FDG avid bony island at right femoral greater trochanter
- FDG biodistribution is rather physiologic, with no significant blastic or lytic skeletal lesions could be seen on CT to account for metastatic disease to bone.





مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الست المصرية
صحة
مصر

MDT FORM 221 2 / 2023 GA

Name	مرفت عبد اللطيف سليم	MRN	٢٠٩٠٧١١١٣...
Age	٥٤	ID	٢٠٩٠٧١١١٣...
Menopausal status	Post	Phone	١١٠٠٢٣٣٢١٩٢
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity	DM - HTN	REASON OF MDT	new

Clinical Data			
BX: LT IDC grade III			
ER: 0/8	PR: 0/8	HER 2: +ve	Ki67: 30%
Sonomammography: Lt UOG 7.0x3.0x5.0 cm with intra ductal extension			
Mets Workup: CT - free			
R L			
TNM cT4d1			

Recommendations:

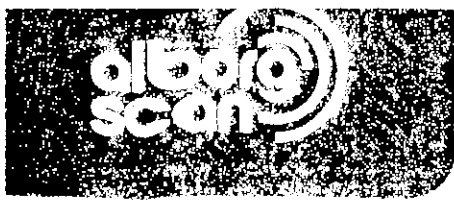
new Sonomammogram at hand: Rt UOG & UOG Fibroadenoma
Rt non specific ALN

Lt T4N2M0
possibility of NACT then Lt Mastectomy

MDT OUTCOME & PLAN

get NACT (Hormonal/Targeted/Herceptin/Regorafenib) to regular clinical assessment. then consider Rt Mastectomy & L prophylactic mastectomy

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



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Visit Number: 61123501743
Patient Name: مبرفت عبد اللطيف سليم
Age / Sex: 64 year / Female
Patient ID: 21 3488004

Client Name: نقابة المهندسين بنسبة - القاهرة
Referred by Prof: -
Exam Date: 22 Jan 2023
Printed: 1/22/2023 2:20:40 PM

US

Clinical data: Check-up.

Transthoracic Echocardiography

LVED	5.3	(3.5 - 5.7 cm)	LA	4.2	(1.9 - 4.0 cm)
LVES	2.7	(2.2 - 4.0 cm)	Ao	2.5	(2.0 - 3.7 cm)
SWT	1.0	(0.7 - 1.1 cm)	RV	2.4	(2.2 - 4.4 cm)
PWT	1.1	(0.7 - 1.1 cm)	MVA	5.2	(2.0 - 4.0 cm ²)
FS	49	(25 - 45 %)	EF	80	(54-82%)

Comments

- **Left ventricle:** Normal left ventricular internal dimensions with good overall global systolic function.
- **Diastolic dysfunction grade II (reversed E/A pattern).**
- There are no segmental wall motions abnormalities can be detected at rest study.
- **Left atrium:** Mildly dilated internal dimensions.
- **Mitral valve:** Thickened and non-calcified mitral valve leaflets with adequate excursion. Mild mitral regurgitation.
- **Aortic valve:** Trileaflet valve, normal leaflets thickness and excursions. No significant systolic gradient is present across the valve.
- **Right sided chambers:** Normal internal dimensions with preserved RV systolic function.
- **Tricuspid valve:** Normal leaflets thickness and excursion, trivial tricuspid regurgitation.
- No evidence of intracardiac masses or thrombi.
- **Pericardium:** Normal thickness, No pericardial effusion.
- **Intact interatrial and interventricular septae** as seen by TTE.
- Normal thoracic aorta.





15 / سرفت عبدالرحمن

ort

MR: ٢٠٩/٢٠٢٣

Name:

Date: 11/10/2023

EF	65%	55-75%	PWT	0.9	
LVEDD	5.3	3.5-5.8cm	AO.R	2.8	2-3.7cm
LVESD		2-4cm	LA	4.1	2-4cm
FS	35%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions Normal

LV wall thickness Normal

LV diastolic function DD grade II

SWMA No SWMA

LA and Aortic root dimensions Normal

Mitral Valve Normal

Aortic valve Sclerotic

Tricuspid valve Mild TR

Pulmonary valve Normal

Right side and systolic function Normal

Intracardiac masses and effusion No

Fair LV systolic function D/

Msc cardiology

EF = 65%
DD grade II

Julia B



Echo report

مرفعة عبد الله بن علي

Name:

Date: 3/7/2024

MR: ٢٥٩

EF	66%	55-75%	PWT	1	
LVEDD	5.4	3.5-5.8cm	AO.R	2.8	2-3.7cm
LVESD		2-4cm	LA	4.3	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	1	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions Normal

LV wall thickness Normal

LV diastolic function DD grade II

SWMA Normal

LA and Aortic root dimensions

LA mildly dilated

Mitral Valve Mild TR

Aortic valve Normal

Tricuspid valve Mild TR

Pulmonary valve Normal

Right side and systolic function Normal

Intracardiac masses and effusion No

D/ Msc cardiology

د/ عيسى



Echo report

عرفت عبد اللطيف

MR359

Name:

Date: 7/3/2024

23

EF	75%	55-75%	PWT	1.3	0.6-1.1cm
LVEDD	5.4	3.5-5.8cm	AO.R	3.6	2-3.7cm
LVESD	2.9	2-4cm	LA	5.8	2-4cm
FS	45	25-45%	RV		0.7-2.3cm(supine)
SWT		0.6-1.1cm	AO.CUSP		1.5-2.6cm

1.25d 1.2

LV dimensions

LV wall thickness

LV diastolic function

SWMA

LA and Aortic root dimensions

Mitral Valve

Aortic valve

Tricuspid valve

Pulmonary valve

Right side and systolic function

Intracardiac masses and effusion

Concentric LVH

E/Em > 1

no RMA

Dilated LA

RVP = 27 mmHg

no

~~Dr. [Name]~~

Msc cardiology

Visit Number:	61123501743	Client Name:	نقابة المهندسين بنسبة - القاهرة
Patient Name:	ميرفت عبد اللطيف سليم	Referred by Prof:	-
Age / Sex:	64 year / Female	Exam Date:	22 Jan 2023
Patient ID:	2113488004	Printed:	1/23/2023 9:00:36 AM

MR

DYNAMIC MR MAMMOGRAPHY**INDICATIONS:** Left breast mass.**TECHNIQUE:**

- Axial T1, TRIM and T2 WIs.
- Sagittal T2 WIs.
- Five axial dynamic contrast-enhanced sequences, after bolus injection of 0.1 mmol/kg of Gadopentetate dimeglumine, using 1.6 mm slice thickness.
- MIP reconstruction.
- Diffusion-weighted images.
- Time/intensity curves created for enhancing lesions.

FINDINGS:**LEFT BREAST:**

- A large; around 7.0 x 3.0 x 5.0 cm (in anteroposterior, transverse and craniocaudal dimensions, respectively), irregular mass is noted, occupying the UOQ of the breast, breast tail and 12 o'clock, extending to the retro-areolar region, with intraductal extension and nipple retraction.



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البرج
سكان

Visit Number: 61123501743
Patient Name: ميرفت عبد اللطيف سليم
Age / Sex: 64 year / Female
Patient ID: 2113488004

Client Name: نقابة المهندسين بنسبة - القاهرة
Referred by Prof: -
Exam Date: 22 Jan 2023
Printed: 1/23/2023 9:00:36 AM

MR

- The forementioned mass has intermediate T1 and T2 signals, with diffusion restriction on DWIs. On dynamic post contrast study, it showed heterogeneous enhancement, with type 3 time/intensity curve (moderate initial contrast uptake followed by early wash-out at 4.11 min).
- Diffuse skin thickening of the breast is seen (7 mm in thickness), with dilated subdermal lymphatics and diffuse interstitial breast edema.
- Pathological axillary level 1 and 2 lymph nodes are noted, few of which showing extracortical extension. Some of the lymph nodes are aggregated together, forming the largest nodal mass which measures around 3.5 x 1.5 cm in maximum dimensions.

RIGHT BREAST:

- Moderate background parenchymal enhancement of the breast (ACR c).
- A 2.0 x 1.5 cm, partially ill-defined, irregular mass is noted at 4-5 o'clock; exhibiting intermediate T1 and dark T2 signals. On dynamic post contrast study, the mass exhibited patchy nodular post contrast enhancement, with type 2 time/intensity curve (high initial contrast uptake, followed by increasing uptake then wash-out at 5.27 min).
- A 1.2 x 1.0 cm, benign-looking, well-defined mass is seen at 7 o'clock, exhibiting intermediate T1 and mostly bright T2 signals, demonstrating rather homogeneous post contrast enhancement and type 1 time/intensity curve; with minimal initial, followed by progressively increasing contrast uptake.
- An axillary lymph node is noted, with blunt hilum, measuring around 2.0 x 1.0 cm, for correlation with ultrasound.

Scan Result



مهندس محمد عبد الله - المهندسين

مهندس محمد عبد الله

مهندس محمد عبد الله

مهندس محمد عبد الله

مهندس محمد عبد الله - BS - الدور الثالث - الشيخ زايد

شارع الجزائر - المعادي

Visit Number: 61123501743
Patient Name: ميرفت عبد الطيف سليم
Age / Sex: 64 year / Female
Patient ID: 2113488004

Client Name: نقابة المهندسين بنسبة - القاهرة
Referred by Prof: -
Exam Date: 22 Jan 2023
Printed: 1/23/2023 9:00:36 AM

MR

OPINION:

- Large left breast mass, with pathological axillary lymphadenopathy, as described (BIRADS V).
- Right breast mass and indeterminate axillary lymph node (BIRADS IV).
- Further assessment by biopsy is recommended.

Thank you for your reference,
Prof. Dr. Fatma Awad,
Cairo University

Scan Result



رياض - برج الاسعة - المهندسين

اغني

رايد

البرج

Name : Mrs. Mervat Abd El Latif Selim MSALME

Date : 25/12/2022

Code : R2823746

3d full field DIGITAL MAMMOGRAPHY AND TOMOSYNTHESIS
EXAMINATION OF BOTH BREASTS:

CLINICAL SUMMARY: 62 yrs old female patient , left breast lump with retracted nipple

IMPRESSION:

RIGHT BREAST (BIRADS 3): multiple probably benign lesions likely fibroadenomata

LEFT BREAST (BIRADS 5): advanced multicentric lesions with highly suspicious micro-calcifications and ipsilateral pathological LNDs level I&II

RECOMMENDATION:

Biopsy and CESM correlation

TECHNIQUE:

Cranio-caudal & Mediolateral Oblique views

High resolution ultrasound

Tomosynthesis in 0.5mm slice thickness

DETAILED FINDINGS:

Breast composition:

The breasts show scattered fibroglandular tissues (ACR b)

Right Breast:

- Multiple well defined oval shaped hyper-dense lesions and scattered macro-calcifications .
- Additional ultrasound revealed normal parenchyma, and shows multiple well defined hypoechoic lesions the largest retro-areolar macro-lobulated measured about 18x12mm
- Normal skin thickness and contour.
- Normal retro-areolar ductal system.
- Normal nipple/areola complex.
- No abnormal axillary lymph nodes.

رئيس مجلس الإدارة
أ. د. مؤمنه كمال

مدير المعامل
أ. د. هند الشربيني

inoamena kamel lab
معمل إنعام كمال

اسم الفرع: تمريض
مدير الفرع: د. نهى فؤاد
مدير الوحدة: أ. د. هالة شيبا
طبيب الوحدة: د. دعاء خميس
د. فتن رمضان

PATIENT NAME مرحلت عبد اللطيف سليم			Registered	24-12-2022 19:50:22	Collected	24-12-2022 19:51:40
			Authenticated	25-12-2022 01:29:06	Printed	25-12-2022 11:20:44
Visit Number	Age	Gender	Referred By		Client ID	
22522518676	64 Year	Female	اسيا عصمت عبد الفتحي : Prof		42984	

Test Name	Result	Unit	Reference Range
Complete Blood Picture			
Haemoglobin (EDTA Blood)	12.1	g/dL	11.5 - 15.5
Haematocrit (PCV)	39.0	%	36 - 45
RBCs Count (EDTA Blood)	4.71	Millions / cmm	4 - 5.2
MCV	82.8	fl	80 - 100
MCH	25.7	pg	27 - 33
MCHC	31.0	g/dl	31 - 37
RDW-CV	14.9	%	11.5 - 15
Platelet Count (EDTA Blood)	235	Thousands / cmm	150 - 450
Total Leucocytic Count (EDTA Blood)	9.3	Thousands / cmm	4 - 11
	<u>Percent Values</u>		<u>Absolute Values</u>
Neutrophils	66.5	%	6.17 x10^9/L
			2 - 7

Name : Mrs. Mervat Abd El Latif Selim MSALME

Date : 25/12/2022

Code : R2823746

Left Breast:

- Most of the breast is seen infiltrated by suspicious polymorphic micro-calcifications as well as retracted nipple and retroareolar architecture distortion and skin thickness
- Upper central well defined oval shaped lesion with internal macro-calcifications
- Additional ultrasound revealed from 2 o clock to 6 o clock zone A&B & C and 6 o clock to 9 o clock zone A &B altered parenchyma with internal suspicious calcifications associated with skin thickness reached about 5mm and subcutaneous edema
- multiple ill defined hypoechoic speculated lesions detected the largest one at 3 o clock deeply seated on the muscle measured about 3.1x2.3cm away from the skin by 3 cm
- retroareolar area of altered echogenicity reached to nipple with architecture distortion .
- at 12 o clock shows well defined oval shaped lesion with internal macrocalcifications measured about 14x9mm
- multiple pathological lymphnodes more than 3 at level one two of them show internal suspicious calcifications the largest measured about 15mm with cortical thickness reached about 6mm and one at level II measured about 9mm .

Sincerely,
Dr. Marwa Abdelhakim ,msc

For any inquiries regarding the findings in the report please contact us on
radreport@cairoscan.com.eg



PATIENT NAME			Registered	24-12-2022 19:50:22	Collected	24-12-2022 19:51:40
مرفت عبد اللطيف سليم			Authenticated	25-12-2022 00:04:40	Printed	25-12-2022 11:20:45
Visit Number	Age	Gender	Referred By		Client ID	
22522518676	64 Year	Female	اسيا عصمت عبد الفتحي : Prof		42984	

Test Name	Result	Unit	Reference Range	Previous Result
Tumor Markers				
CA 125	5.88	U/mL	0 - 35	



A. Nokaiy
Dr.ABD El-Aziz El-Nokaiy
Professor of Medical Bio-Chemistry,
Faculty of medicine, EL Azhar University 1-1

MRS: Mervat Abd Al Latif Selim	Date: 2023/07/18
Dear. SIR.	ID: 928155

**BILATERAL DIGITAL COMPUTED MAMMOGRAPHY AND SONOGRAPHY
REVEALED:-**

Known patient of left breast carcinoma on neo adjuvant CTH, the current study revealed:

The right breast:

- The right breast shows normal nipple, areola, skin and subcutaneous tissue thickness.
- It shows condensation of glandular tissue elements, with nodularity, suggestive of fibroadenosis.
- The right breast shows paraareolar inner quadrant 4 O'clock position lobulated opacity **by ultrasonography** it proved to be lobulated soft tissue lesion at 5 O'clock position measuring 17 x 10 mm, mostly represent fibroadenoma.
- The right breast shows few simple cysts measuring 3 x 2 mm.
- No suspicious breast masses are detected.
- No malignant micro calcifications are detected.
- No suspicious right axillary lymphadenopathy.

The left breast:

- The left breast shows mild diffuse skin and subcutaneous soft tissue thickening and edema.
- It shows condensation of glandular tissue elements, with nodularity, suggestive of fibroadenosis.
- It shows outer quadrant diffuse suspicious micro calcifications, **by ultrasonography** it proved to be area of underlying fibrosis and architectural distortion with suspicious mass lesion at 2 O'clock position now it measures 16 x 9 mm (for comparison with the previous study).

ا.د. أحمد كمال البدرى
ا.د. محمد أبو الهدى درويش
ا.د. عمر حسين عمر
ا.د. وحيد حسين طنطاوى
أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

مركز مصر للأشعة
MISR RADIOLOGY
CENTER



MRS: Mervat Abd Al Latif Sellm	Date: 2023/07/18
Dear. SIR.	ID: 928155

- The left breast shows upper central retroareolar benign looking opacity with benign looking coarse calcifications inside measuring 13 x 8 mm.
- Left enlarged axillary lymph node with foci of micro calcifications inside and deeply hypoechoic echotexture, largest one measuring 9 x 5 mm.

CONCLUSION:-

- Left breast outer quadrant suspicious mass lesion with dimensions as described (for comparison with the previous study) (BIRADS: 6) on neo adjuvant CTH.
- Bilateral fibroadenosis with bilateral benign looking lesion represent fibroadenoma (BIRADS: 3).

MUCH OBLIGED
DR. SHAIMAA EL DIASTY, MD
PROF. DR. OMAR HUSSEIN
A.G.

د. أحمد كمال الدري
د. محمد أبو الهدى درويش
د. عمر حسين عمر
د. وحيد حسين طنطاوى
أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

MRS: Mervat Abd Al Latif Selim	Date: 2023/07/18
Dear. SIR.	ID: 928155

BI-RADS 0	Need Additional Imaging Evaluation and/or Prior Mammograms For Comparison:
BI-RADS 1	Negative:
BI-RADS 2	Benign Finding:
BI-RADS 3	Probably Benign Finding - Initial Short-Interval Follow-Up Suggested: less than a 2% risk of malignancy.
BI-RADS 4	Suspicious Abnormality - Biopsy Should Be Considered: wide range of probability of malignancy (2 - 95%).
BI-RADS 4 Category 4a	low suspicion for malignancy
BI-RADS 4 Category 4b	moderate suspicion for malignancy
BI-RADS 4 Category 4c	high suspicion for malignancy
BI-RADS 5	Highly Suggestive of Malignancy. Appropriate Action Should Be Taken: with a >95% likelihood of malignancy.
BI-RADS 6	Known Biopsy Proven Malignancy. Appropriate Action Should Be Taken

ACR Classification of breast density

- 1 = Almost entirely fatty
- 2 = Scattered areas of fibro-glandular density
- 3 = Heterogeneously dense
- 4 = Extremely dense



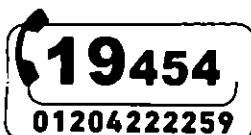
Req.#:- 8849373 Patient No : 55739-8
Name:- الاستاذة / مبرفت عبد اللطيف سليم Sex:-Female Age:- 65 Y Birthdate
Collection Date: 24/07/2023 17:16 Reporting Date:- 25/07/2023 19:09
Contract:- نقابة المهندسين Referred By:-

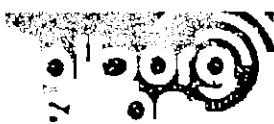
IMMUNOLOGY

Test	Result	Ref.Range
HBs Ag.	Negative	Negative
HCV IgG - 3rd generation	Negative	Negative

Branch Manager

Dr. om al





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Visit Number: 61123501743

Client Name: نقابة المهندسين بنسبة - القاهرة

Patient Name: ميرفت عبد اللطيف سليم

Referred by Prctf: -

Age / Sex: 64 year / Female

Exam Date: 22 Jan 2023

Patient ID: 2113488004

Printed: 1/22/2023 2:20:40 PM

US

Clinical data: Check-up.

Transthoracic Echocardiography

LVED	5.3	(3.5 - 5.7 cm)	LA	4.2	(1.9 - 4.0 cm)
LVES	2.7	(2.2 - 4.0 cm)	Ao	2.5	(2.0 - 3.7 cm)
SWT	1.0	(0.7 - 1.1 cm)	RV	2.4	(2.2 - 4.4 cm)
PWT	1.1	(0.7 - 1.1 cm)	MVA	5.2	(2.0 - 4.0 cm ²)
FS	49	(25 - 45 %)	EF	80	(54-82%)

Comments

- **Left ventricle:** Normal left ventricular internal dimensions with good overall global systolic function.
- **Diastolic dysfunction grade II (reversed E/A pattern).**
- There are no segmental wall motions abnormalities can be detected at rest study.
- **Left atrium:** Mildly dilated internal dimensions.
- **Mitral valve:** Thickened and non-calcified mitral valve leaflets with adequate excursion. Mild mitral regurgitation.
- **Aortic valve:** Trileaflet valve, normal leaflets thickness and excursions. No significant systolic gradient is present across the valve.
- **Right sided chambers:** Normal internal dimensions with preserved RV systolic function.
- **Tricuspid valve:** Normal leaflets thickness and excursion, trivial tricuspid regurgitation.
- No evidence of intracardiac masses or thrombi.
- **Pericardium:** Normal thickness, No pericardial effusion.
- **Intact interatrial and interventricular septae** as seen by TTE.
- Normal thoracic aorta.



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Visit Number: 61123501743

Patient Name: ميرفت عبد اللطيف سليم

Age / Sex: 64 year / Female

Patient ID: 2113488004

Client Name: نقابة المهندسين بنسبة القاهرة

Referred by Prof: -

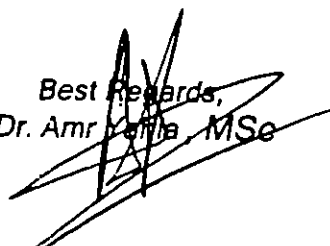
Exam Date: 22 Jan 2023

Printed: 1/22/2023 2:20:40 PM

US

Conclusion:

- Degenerative heart disease with preserved systolic function.
- Impaired LV relaxation.
- Mild MR.
- Dilated LA.

Best Regards,
Dr. Amr  MSc





٢٠٩

٢٢

Echo report

Name:

Date : 10/5/2023

EF	66%	55-75%	PWT	1.0	0.8-1.1cm
LVEDD	5.0	3.5-5.8cm	AO.R	2.7	2-3.7cm
LVESD	3.2	2-4cm	LA	4.1	2-4cm
FS	37%	25-45%	RV		0.7-2.3cm(supine)
SWT	1.1	0.6-1.1cm	AO.CUSP		1.5-2.6cm

systolic function with internal dimension EF 66 %

diastolic function. grade I

Mildly dilated LA and AO root dimensions.

Mitral Valve morphology, Mild Central MR.

No resting SWMA.

Elastic Aortic valve morphology, No gradient.

Tricuspid valve morphology and leaflets

Normal Pulmonary Valve.

No intracardiac masses or pericardial effusion

Conclusion: Mild MR with Mildly dilated LA.

Diastolic dysfunction grade I.

٢٢

Dr. Mohamed A. Wahab

Patient Name : الأستاذ / مرفت عبداللطيف سليم	Request No. : 13009106
Patient No. : 25907111300062	Request Date : 02/02/2023
Patn No. : 326/2023	Verification Date : 11/02/2023
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 63 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Known case of left breast cancer with right breast mass (BIRAD4).

Nature of the specimen: Tru-cut biopsy of right breast lesion. collected at 1/2/2023, 3:40 PM.

Gross Examination:

Four graigh white tissue cores, 1 cm longest core, totally submitted.

Microscopic examination:

Histopathologic examinations of prepared slides reveals breast tissue cores with ductectasia, focal papillomatosis, adenosis, fibroadenoma, and fibrosis.

Diagnosis:

Right breast mass; Tru-cut biopsy:

- Fibrocystic changes
- No malignancy

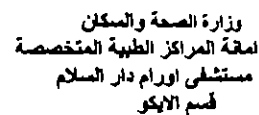
Prof.Dr. Iman Gouda
Dr.Heba mostafa

Verified by :
walaa ghoniem

Printing Date : 11/02/2023 14:50:34

Page : of 1

Printing By : marwa mohamed youss



MR: 709/127

Date: 7/7/2023

LV dimensions *normal*
 LV wall thickness *normal*
 LV diastolic function *DD*
 SWMA *no DWMA*
 LA and Aortic root dimensions *normal, dilated LA*
 Mitral Valve *mild MR*
 Aortic valve *sclerotic but significant regurgitant*
 Tricuspid valve *mild TR* *CPAP ~ 20 mmHg*
 Pulmonary valve *normal*
 Right side and systolic function *normal*
 Intracardiac masses and effusion *no masses, no effusion*

D/ Msc cardiology



وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "
إدارة الجودة

أوامر الطبيب Doctor's Orders

إسم المريض ثلاثي: مرفقة بجواز الطبيب سليم

رقم الملف :

اسم المريض ثلاثي:

رقم الملف:

1H 153

wt 91

1.0 1.9



وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة

مستشفى أورام دار السلام " هرمل "
إدارة الجودة

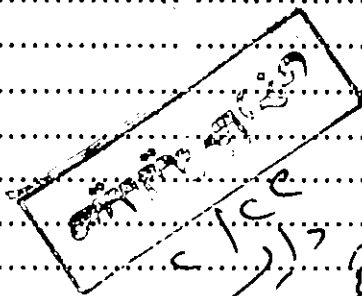


80%

Doctor's Orders أوامر الطبيب

التشخيص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		* Known case of Adenocarcinoma of the stomach. C T Ad N. 2 M. * ER 0.18 PR 0.18 Hx2 + v. K. 17 3 = 1. A NA TCHIP Taxol 750 mg IV 142 Cisplatin AUC 5 Methotrexate 25 1x2x6 Carboplatin 400 mg 1x1x21 Granisetron 2mg IV 1x1x5 HP noted until final report	
		22/12/23	



دار السلام
مركز الأورام
إدارة الجودة

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام (هرمل)
الصيدلة الإكلينيكية

SA
1.96

TCHP (with weekly Paclitaxel) cycle (.....) ^{W1} + 2

اليوم: التاريخ: 2021/...../.....		اسم المريض: ABW: 64 kg	
رقم الملف:	Weight: 64 kg	Height: 153 cm	BSA: 1.96 M2
CBC:	WBC:	ANC:	Hgb: PLT:
Functions Tests:	Sr.Creat:	Urea:	CrCL:
	ALT:	AST:	T.Bil: D.Bil:
Echo:	Date:	EF: .. %	
Dose Modification: %			
Reason: ☐ Hematological ☐ Non hematological:			
الصرف	بروتوكول العلاج		
الاسبوع 1	الاسبوع الاول: جرانيسترون (0.01 مجم/كجم) مجم + ديكساميثازون 20 مجم + انتودين 20 مجم + امبول افيل) في 50 مل ج 5% علي مدي ربع ساعة قبل العلاج الكيمائي. (Maximum dose of Granisetron = 1mg)		
الاسبوع 2	الاسبوع الثاني: باكليتاكسل (80 مجم/2م) مجم في 250 مل ج 5% علي مدي ساعة.		
الاسبوع 3	الاسبوع الثالث: كاربوبيلاتين (AUC 2) مجم علي مدي ساعة.		
يعطي بتاريخ	تراستوزوماب 600 مجم تحت الجلد لمدة 5 دقائق في الفخذ.		
..... / /	برتوزوماب مجم في 250 مل ملح علي مدي 1/2 ساعة (علي مدي ساعة في الجرعة الاولى) Loading Dose: 840 mg Maintenance Dose: 420 mg		
العلاج التدعيمى:	كونترولوك قرص 40 مجم علي الريق يوميا. ديكساميثازون 8 مجم (امبول او قرص) مرة يوميا اليوم الثاني و الثالث. ☒ جرانيسترون قرص 1 مجم كل 12 ساعة اليوم الثاني و الثالث. ☒ اوندالينز 8 مجم قرص كل 12 ساعة اليوم الثاني و الثالث.		
Physician Signature:	Clinical pharmacy: W 1 >> Doan B Tc (145mg) W 2 >> Doan B Tc (145mg) W >> Trastuzumab + pertuzumab > Date		
	Dispensing pharmacy: 25/2/2023 4/3/2023 مستشفى اورام دار السلام		

Taxol + الادرال دار اورام
2023/2/25



وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "
إدارة الجودة

أوامر الطبيب
Doctor's Orders

إسم المريض ثلاثي:
رقم الملف:
Breast Ca

رقم الملف:

Doctor's Orders اوامر الطبيب

توقيع الطبيب	الأوامر	الوقت	التاريخ
	64 yrs ♀ LE LABC ER-ve, PR-ve, Her2 +ve CT4 N2 MDT → neoadj Taxol/carbo H-P x 6 cycles then LE MDM -		

Page 1 of 1

إسم المريض ثلاثي:
رقم الملف:



وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
مستشفى أودام دار السلام " هرمل "
إدارة الجودة

153
91

Doctor's Orders أوامر الطبيب

التشخيص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		1. ... 2. ... 3. ... 4. ... 5. ... 6. ... 7. ... 8. ... 9. ... 10. ... 11. ... 12. ... 13. ... 14. ... 15. ... 16. ... 17. ... 18. ... 19. ... 20. ... 21. ... 22. ... 23. ... 24. ... 25. ... 26. ... 27. ... 28. ... 29. ... 30. ... 31. ... 32. ... 33. ... 34. ... 35. ... 36. ... 37. ... 38. ... 39. ... 40. ... 41. ... 42. ... 43. ... 44. ... 45. ... 46. ... 47. ... 48. ... 49. ... 50. ... 51. ... 52. ... 53. ... 54. ... 55. ... 56. ... 57. ... 58. ... 59. ... 60. ... 61. ... 62. ... 63. ... 64. ... 65. ... 66. ... 67. ... 68. ... 69. ... 70. ... 71. ... 72. ... 73. ... 74. ... 75. ... 76. ... 77. ... 78. ... 79. ... 80. ... 81. ... 82. ... 83. ... 84. ... 85. ... 86. ... 87. ... 88. ... 89. ... 90. ... 91. ... 92. ... 93. ... 94. ... 95. ... 96. ... 97. ... 98. ... 99. ... 100. ...	

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

..... إسم المريض ثلاثي:
..... رقم الملف:

EF, 80% (22/112023).

Doctor's Orders **اوامر الطبيب**

.....: نوع التغذية : الشخص:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		Patient experienced skin reaction following 1st cycle. So please for ↑ Dexamethasone dose with monitoring blood sugar - & dose reduction of Taxol & Carboplatin ديكساميثازون 16 مجم عند الاستيقاظ صباحاً W2 تاكسول 100 مجم AUC 1.5 هرسنة 100 مجم تحت الجلد بزل 1X1X1 مستلزمات OXOX برص 100 مجم دالاسون 14/3/2023	4/3/2023

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

C1W3TC110mg
11/3/2023
PH/AHMED

.....: اسم المريض ثلاثي:
.....: رقم الملف:

Doctor's Orders **اوامر الطبيب**

التشخيص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		- Last chemotherapy tolerated after dose reduction. - C/O oral mucositis G1. - Labs pending, if ok per: C2 W1 السجل ١٠٥٣ AUC 1.5 كاربوفازين X X X ١ بنود O X 1 X ١ محم ديفو كابر O X C X ١ اوتو البند O X C X ١ جيتيل O X 1 X ١ كلاريت داري زخم C-CX ١٢/١١ C2 W1 T.C 110 11/3/2023 ph/AHMED	

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

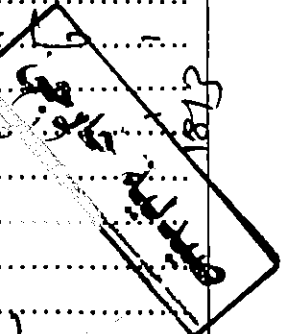


اسم المريض ثلاثي:
رقم الملف:

Doctor's Orders

نوع التغذية: يص:

توقيع الطبيب	الأوامر	الوقت	اريخ
c/o. diarrhea GI	يعمل العلاج الكيماوي بأقصى جرعة ممكنة AUC 2 12 x 2 x 1 V x 2 x 1 V x 1 x 1 V x 2 x 1 C cuc 1 x 1	W4	
after consulting Dr/Mostafa Auc 2 ≠ Auc 1.5	كاربوبيدين 12 x 2 x 1 V x 2 x 1 V x 1 x 1 V x 2 x 1 C cuc 1 x 1		
subject implant	cph/Dose B Wg TC (1.45mg) 18/3/2023		
	12 x 2 x 1 V x 2 x 1 V x 1 x 1 V x 2 x 1 C cuc 1 x 1	W5	2
	12 x 2 x 1 V x 2 x 1 V x 1 x 1 V x 2 x 1 C cuc 1 x 1	W6	1
	12 x 2 x 1 V x 2 x 1 V x 1 x 1 V x 2 x 1 C cuc 1 x 1	W7	8



ميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

WTC [138mg]
+ C2 Pharygo
Cl.ph/Absor
SMC DSH-F-Phy
Revision no: 2
Issue no: 3
Issue Date: 1/7/2022

12 x 2 x 1
V x 2 x 1
V x 1 x 1
V x 2 x 1
C
cuc 1 x 1

Page of
150

د/ال
12/12/20

cph/Dose B
Wg TC (138mg) 1/4/2023

WTC [138mg]
8/4/2023 Cl.ph/Absor

.....: اسم المريض ثلاثي:
.....: رقم الملف:

Doctor's Orders **اوامر الطبيب**

.....: **التشخيص:** **نوع التغذية:**

توقيع الطبيب	الأوامر	الوقت	التاريخ
	CB 8.11 AH (44) Fr W W W 8 9 10 C 3 HPD Subjective local response. MALWA RY/8/10		11/11

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام (هرمل)
الصيدلة الإكلينيكية

Paclitaxel Weekly Protocol (weeks & &)

اليوم:		التاريخ: 2021 / /	
أسم المريض: <u>مريم عبد الصنف</u>		رقم الملف:	
Weight: <u>71</u> ... kg	Height: <u>153</u> .. cm	BSA: <u>1.96</u> M2	Age: <u>64</u> years
CBC:	WBC: ANC: Hgb: PLT:		
Functions Tests:	Sr.Creat: Urea: CrCl:		
	ALT: AST: T.Bil: D.Bil:		
Dose Modification: %			
Reason: ☐ Hematological ☐ Non hematological:			
امضاء التمريض	بروتوكول العلاج		الصرف
	جرانيسترون (0.01 مجم/كجم) مجم + ديكساميثازون 20 مجم + انتودين 20 مجم + امبول افيل) في 50 مل ج 5% علي مدي ربع ساعة قبل العلاج الكيماوي. (Maximum dose of Granisetron =1mg)	الاسبوع <u>8</u> بتاريخ <u>10/12/2021</u>	
	باكليتاكسل (80 مجم/م) مجم في 250 مل ج 5% علي مدي ساعة.	الاسبوع <u>9</u> بتاريخ <u>19/12/2021</u>	
	50 مل ج/ملح علي مدي 10 دقائق	الاسبوع <u>10</u> بتاريخ <u>27/12/2021</u>	
كورتيزون قرص 40 مجم علي الطريق يوميا كورل 100 مل ديكساميثازون 20 مجم ديكساميثازون 20 مجم ديكساميثازون 20 مجم		العلاج الكيماوي الاسبوع <u>11</u> بتاريخ <u>27/12/2021</u>	
Physician Signature:	Clinical pharmacy:	Dispensing pharmacy:	
	W W W TC 180 ⇒ 29/4/2023 ⇒ ph/AHMED	W8TC [177g] +C3 phygo Cl.ph/AHMED 15/4/2023	

التاريخ: 2021 / /		اليوم:	
رقم الملف:		اسم المريض: <u>مروة عبد اللطيف سالم</u>	
Weight: kg	Height: cm	BSA: M2	Age: years
CBC:	WBC: ANC: Hgb: PLT:		
Functions Tests:	Sr.Creat: Urea: CrCl:		
	ALT: AST: T.Bil: D.Bil:		
Echo:	Date: EF: %		
Dose Modification: %			
Reason: ☐ Hematological ☐ Non hematological:			
امضاء التمريض	بروتوكول العلاج		الصرف
الاسبوع الاول:	جرانيسيترون (0.01 مجم/كجم) مجم + ديكساميثازون 20 مجم + انتودين 20 مجم + امبول افيل في 50 مل ج 5% على مدي ربع ساعة قبل العلاج الكيميائي. (Maximum dose of Granisetron = 1mg)	الاسبوع 1 ✓ بتاريخ 12/5/23 W12	مروة عبد اللطيف سالم
الاسبوع الثاني:	باكليتاكسل (80 مجم/ل) مجم في 250 مل ج 5% على مدي ساعة.	الاسبوع 2 بتاريخ 19/5/23 W13	
الاسبوع الثالث:	كاربوبلاتين (AUC 2) مجم على مدي ساعة.	الاسبوع 3 بتاريخ 26/5/23 W14	
1	تراستوزوماب 800 مجم تحت الجلد لمدة 5 دقائق في الفخذ.	يغطي بتاريخ / /	مروة عبد اللطيف سالم
	برتوزوماب مجم في 250 مل ملح على مدي 1/2 ساعة (على مدي ساعة في الجرعة الاولى) Loading Dose: 840 mg Maintenance Dose: 420 mg	5	
العلاج التدايمي: كونترولوك قرص 40 مجم على الريق يوميا ديكساميثازون 8 مجم (امبول او قرص) مرتين يوميا اليوم الثاني و الثالث. ☐ جرانيسيترون قرص 1 مجم كل 12 ساعة اليوم الثاني و الثالث. ☐ اونديالينز 8 مجم قرص كل 12 ساعة اليوم الثاني و الثالث.			
Physician Signature: <u>د. رانيا محمد</u>		Clinical pharmacy: W14 Trastuzumab (145mg) Eph10 W15 W16 Trastuzumab + pertuzumab > Date	
		Dispensing pharmacy: W13 13/5/2023	

إسم المريض ثلاثي:
رقم الملف:



وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "
إدارة الجودة

Doctor's Orders الأوامر الطبية

التشخيص: نوع التغذية:

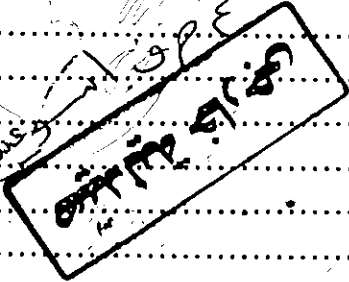
التاريخ	الوقت	الأوامر	توقيع الطبيب
		W. 10 + HP 24 W. 10 TC [178mg] C 24 0.17 + C. phlego Cl. ph 1A beer 6/5/2023 13/5/23 W. 10 TC أنا كوني - ألي Avec 2, 10 7000 كوني 10	

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي: محمد - عبد الله - سليم
رقم الملف: ٢٠٩٩/٤٤٤٤

Doctor's Orders **اوامر الطبيب**

.....: نوع التقنية: التشخيص

التاريخ	الوقت	الأوامر	توقيع الطبيب
Age 64 wt 153 ht 91 BSM 1.96 ABwt 64 hypertension Taxol + Carboplatin 150 mg Ceph Sant O/C.		w ₁₂ w ₁₃ 27/5 C5 AD → 2114 o/e good clinical w ₁₂ C6 phesga + w ₁₃ TC145 27.1.5.1. 2023 ph / AHMED MAHER	د. أحمد السعيد ٢٣/٥/٢٠٢٣  د. أحمد السعيد ٢٧/٥/٢٠٢٣

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

Doctor's Orders **أوامر الطبيب**

[illegible]

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

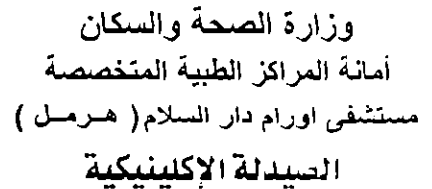
وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام " هرمل "
إدارة الجودة

Doctor's Orders اوامر الطبيب

.....: **التشخيص:**: **نوع التغذية:**

التاريخ	الوقت	الأوامر	توقيع الطبيب
		T2 ١٦.٧.٢٠٢٤ د. محمد ٢٥٨٦ (م) N+8 ١٧/٧/٢٠٢٤ في يوم الاثنين الموافق ١٧ من شهر ربيع الثاني سنة ١٤٤٦ هـ د. محمد فاروق	

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب



اليوم:		التاريخ: 2021 / /	
اسم المريض: السيد عبد الله بن علي		رقم الملف: A.B.W.T. 64	
Weight: 91 kg	Height: 153 cm	BSA: 1.96 M2	Age: 64 years
CBC:	WBC:	ANC:	Hgb: PLT:
Functions Tests:	Sr.Creat:	Urea:	CrCl:
	ALT:	AST:	T.Bil: D.Bil:
Dose Modification: %			
Reason: ☐ Hematological ☐ Non hematological:			
الصرف	بروتوكول العلاج	امضاء التمريض	
الاسبوع W. 7	جرانيسترون (0.01 مجم/كجم) مجم + ديكساميثازون 20 مجم + انتودين 20 مجم + امبول افيل في 50 مل ج 5% على مدي ربع ساعة قبل العلاج الكيماوي. (Maximum dose of Granisetron = 1mg)		
بتاريخ ٢٤ / ٦ / ٢٠٢١			
الاسبوع W. 8			
بتاريخ ٨ / ٧ / ٢٠٢١			
الاسبوع	باكليتاكسل (80 مجم/م) ٥٠٠. مجم في 250 مل ج 5% على مدي ساعة.		
بتاريخ / /			
	50 مل ج/ملح على مدي 10 دقائق		
العلاج التداخلي: كوتنرولوك قرص 40 مجم على الريق يوميا. دليلي - 3 - بوب فلاصين - 3 - مل سوت كل 3 - ملاح 3 - بوب			
Physician Signature:		Clinical pharmacy:	
		W 17-7 C (146mg) ٢٤ / ٦ / ٢٠٢١ W 18-7 C 145 ٢٤ / ٧ / ٢٠٢١ W >>	
Dispensing pharmacy:			

.....: إسم المريض ثلاثي:
.....: رقم الملف:

Doctor's Orders **اوامر الطبيب**

التشخيص:..... نوع التغذية:.....

توقيع الطبيب	الأوامر	الوقت	التاريخ
	MRU → PCR for agj HP 1 yr. 7-7-2023 EF → BS.1 24/7/2023 C7 phesgo 21/9/2023 ص. ا. ح. ا. ب. د. ي		
23/9	60/60 C8 phesgo ص. ا. ح. ا. ب. د. ي		

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب
c. ٢٢/ ٩/ ٢٢

..... رقم الملف:



وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "
إدارة الجودة

Doctor's Orders **اوامر الطبيب**

.....: نوع التقنية:

[illegible]

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

..... رقم الملف:



وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "
إدارة الجودة

GF: 65% (11/10/20²³)

Doctor's Orders **اوامر الطبيب**

.....: **التشخيص:** **نوع التغذية:**

التاريخ	الوقت	الأوامر	توقيع الطبيب
4/11/2019	Clo	Su end ph 4e ay 11 sh x su end SAL 3019 end 18/10 <u>المصدر القوي</u>	w
4/11/2019	Clo phesgo a.pn/ r'Gous	600/600 - - Su end ph 4e ay 11 sh x su end - SAL 3019 end 18/10 - المصدر القوي	w
4/11/2019	Hb 10 Tic 6.1 AD 256 AST 2 ALT 2L Su 1.5 UA 8.1 cre 71	600/600 - - Su end ph 4e ay 11 sh x su end - SAL 3019 end 18/10 - المصدر القوي	25/11/19

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

..... رقم الملف:



Doctor's Orders **اوامر الطبيب**

.....: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
C12		مريض بمرض السكري / 600/600 مريض بمرض السكري / 600/600 مريض بمرض السكري / 600/600 مريض بمرض السكري / 600/600 مريض بمرض السكري / 600/600	16/12/2019
C12 phesgo	صباح / مساء		
C-xx/17/17			
C13 P1			

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

C13 phesgo
6/1/2023
الم

تم صرف فيلصو
C13 + بالامول
45.00
ب. الكرم
C. 21/1/7

.....: **رقم الملف:**



Doctor's Orders **اوامر الطبيب**

..... نوع التغذية :

[illegible]

لديه لاد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:
رقم الملف: ١٧١٢١٤



وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
مستشفى أودام دار السلام " هرمل "
إدارة الجودة

Doctor's Orders الأوامر الطبيب

التشخيص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
15		17/12/2024 Cis Pheobo CLPh / ١٧١٢١٤ (17/12/2024)	

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

..... رقم الملف:



وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "
إدارة الجودة

Doctor's Orders **اوامر الطبيب**

.....: نوع التقنية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
7/3/2024	FF = 75%	[7/3/2024 FF = 75%] → 7/3/2024	
		- مريض - - أدوية - - فحص - - علاج -	
		on EF	
		C16 phetgo صاحب العمل C. ٢٠١٩	
		طبيب - - ٢٠١٩	
		٧/٣	
		وأتى إلى	

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب



.....: اسم المريض ثلاثي:
.....: رقم الملف:

Doctor's Orders **اوامر الطبيب**

.....: نوع التغذية:: التشخيص:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		<u>نصف</u> - Nitrofurantoin 60mg 1/12 كل 12 ساعة - Cymbartek 60mg 1/12 كل 12 ساعة	د/ محمد 14/12

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

رقم الملف: ٢٥٩/٤٣

Doctor's Orders **اوامر الطبيب**

.....: نوع التغذية: التخصيص:

[illegible]

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

